

NO EXCUSES

A Real Person's Guide to
Metabolic Health

By No Excuses Jesse

LoadLine Fitness & Wellness

Queen Creek, Arizona

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DEDICATION

For everyone who's ever sat in the dark, too ashamed to ask for help.

For everyone who's been told "just eat less and move more" by someone who's never been where you are.

For everyone who's tried and failed, and tried again, and failed again, and wondered if the next try would be different.

This book is proof that it can be.

And for my team at LoadLine Fitness, who believed in this work before there was anything to show for it. Miah, Ruth, Markus, Octavia, Mitch, and Parris, you made this possible.

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For the people who follow No Excuses Jesse on social media and share their own stories in the comments. You keep me honest. You keep me going. And you remind me every day why this

work matters.

For my buddy who dragged me to Thailand, even though I didn't want to go. You didn't know it at the time, but that trip changed my life. Not because it was beautiful or fun, but because it showed me exactly where I was. Sometimes you need to see the truth before you can start fixing it.

And for anyone reading this who's at the beginning, who's standing at the bottom of a mountain they can't see the top of, who's wondering if it's even worth trying. It is. You are. Keep going.

AUTHOR'S NOTE

I wrote this book over the course of several months, usually early in the morning before the rest of the world was awake, and usually with a cup of black coffee and a protein shake on the desk next to me. Some mornings the words came easily. Other mornings I sat staring at a blank screen, wondering if any of this mattered.

It matters. I know it matters because I've lived it. Every word in this book is something I either did, learned, failed at, or figured out the hard way. There's nothing here that came from a textbook I didn't eventually cross-reference with my own experience. There's nothing here that I'm selling you that I haven't tested on myself first.

I'm not a perfect writer. I'm not a perfect coach. I'm definitely not a perfect health specimen. But I am honest, and in a fitness industry that runs on curated highlight reels and cherry-picked before-and-after photos, honesty might be the most valuable thing I can offer you.

If this book helps even one person avoid the 2 AM panic attack in the dark, the Thailand humiliation, the years of wasted money on supplements that did nothing, the slow-motion collapse of a body that was screaming for help and getting ignored, then every early morning and every difficult chapter was worth it.

Thank you for reading. Now go do the work.

No excuses.

A FINAL WORD ON CONSISTENCY

I want to leave you with one last thought, because it's the one that matters most and the one that's easiest to forget.

Consistency is not the same as perfection.

Perfection says you eat clean every day, hit your step count every day, sleep eight hours every night, never miss a workout, never eat fast food, never have a bad day. Perfection is a fantasy. It doesn't exist. And chasing it is how most people end up quitting, because the moment they fail to be perfect, they decide they've failed entirely.

Consistency says you do the right things most of the time. You eat protein at most meals. You walk most days. You sleep well most nights. You strength train most weeks. You get your bloodwork done on schedule most quarters. And when you miss, because you will miss, you pick it back up at the next opportunity instead of waiting for next Monday, next month, next year.

I've been doing this for over a year now. I've had weeks where I didn't walk. I've had days where I ate nothing but garbage. I've had nights where I got four hours of sleep and woke up feeling like I'd been hit by a truck. I've had moments where I wanted to quit all of it, the tracking, the training, the protein, the whole system, and just go back to eating whatever I wanted whenever I wanted.

I didn't quit. Not because I'm special. Not because I have more willpower than you. I didn't quit because I'd built a system that made quitting feel worse than continuing. The streak was real. The data was real. The progress was real. And when I looked at what I'd built, even on my worst days, I didn't want to throw it away.

That's what consistency gives you. It gives you something worth protecting. It gives you evidence that you're the kind of person who shows up, even imperfectly, even inconsistently, even on the days when everything feels wrong. And over time, that evidence becomes identity. You stop

being a person who's trying to get healthy and start being a person who is healthy. The verb becomes a noun. The action becomes identity.

That's the real transformation. Not the weight loss. Not the bloodwork numbers. Not the smaller shirt size. The real transformation is becoming someone who does hard things, consistently, without waiting for motivation or perfect conditions.

You can become that person. You already are that person. You just need to prove it to yourself, one day at a time, one habit at a time, one walk at a time.

No excuses.

INTRODUCTION

The Real Version

This book started as notes on my phone. Scribbled reminders after meals, step counts from my watch, protein grams scratched onto the back of receipts. I wasn't writing a book. I was surviving. I was trying to figure out why every diet I'd ever tried had failed, why the weight always came back, and why nobody in the fitness industry seemed willing to talk about the actual experience of being a person who struggles with their health.

I'm not a doctor. I'm not a nutritionist. I don't have a certification on my wall or a lab coat in my closet. What I have is a story, a system that actually worked, and the stubbornness to write it down honestly.

Here's the short version: I was 317 pounds. Then 340. Then 368. I was diabetic, hypertensive, and sleeping so badly that I woke up gasping for air most nights. I couldn't walk to my mailbox without needing to rest. I couldn't fit comfortably in a restaurant booth. I couldn't get on an airplane without asking for a seatbelt extension. I was 51 years old, and my body was failing in ways I was too ashamed to talk about.

Then I fixed it. Not overnight. Not with a magic pill (though medication played a role, and I'll be honest about that too). Not with a personal chef, a home gym, or unlimited free time. I fixed it the hard way, one habit at a time, using four pillars that I now know are backed by decades of research I didn't have access to at the time.

Those four pillars are:

NUTRITION: Not a diet. Not a cleanse. A way of eating that prioritizes protein, whole foods, and enough fiber to keep you full and your blood sugar stable.

MOVEMENT: Not a marathon. Not a CrossFit class. Daily walking, starting with whatever you can manage, building gradually toward a target that changes your metabolic health.

SLEEP: Not eight hours of perfect rest. Just better sleep, consistently, with a routine that tells your body it's safe to rest.

NUMBERS: Not guessing. Not hoping. Tracking what matters, protein, steps, sleep, and bloodwork, so you can see what's actually happening inside your body instead of relying on how you feel.

That's it. Four pillars. No secrets. No shortcuts. No "one weird trick." Just the boring, unglamorous, actually effective work of rebuilding your health from the ground up.

Who This Book Is For

This book is for anyone who's tired of being sold solutions that don't work. It's for the person who's tried keto, carnivore, intermittent fasting, Weight Watchers, juice cleanses, and a dozen other approaches that worked for a while and then didn't. It's for the person who's been told "just eat less and move more" by someone who's never been 100 pounds overweight. It's for the person who knows what they should be doing but can't seem to make it stick.

It's also for the person who's considering a GLP-1 medication, Ozempic, Wegovy, Mounjaro, Zepbound, and wants an honest account of what that experience is actually like, not the sanitized version from a pharmaceutical ad or the horror story from someone who used it wrong.

And it's for the person who just got bad news from their doctor and doesn't know where to start. If that's you, start here. Start with Chapter 1. I'll walk you through it.

The LoadLine Framework

Everything in this book is organized around what I now call the LoadLine standard: TRAIN,

MOVE, RECOVER, TRACK, REPEAT.

TRAIN: Build strength. Three sessions a week, using bands, bodyweight, or a gym. Muscle is your metabolic engine.

MOVE: Walk. Every day. After meals. Build toward 7,000 to 10,000 steps. Walking is the most underrated health intervention in existence.

RECOVER: Sleep. Seven to nine hours. A wind-down routine. Address sleep apnea if you have it. Sleep is where your body repairs itself.

TRACK: Log your numbers. Nutrition, workouts, steps, body measurements, and bloodwork. Data beats guesswork.

REPEAT: Do it again. Every day. Every week. Every month. Consistency is the only variable that matters long term.

This framework isn't mine. I didn't invent it. I discovered it through months of trial and error, and then I found out that the research supports every piece of it. I just gave it a name and a structure that made it easier to follow.

The LoadLine Tracker, available at loadlinefitness.com/tracker, was built around this exact framework. You don't need it to use this book, but if you want a tool that tracks all five domains in one place, it's there.

A Note on Honesty

I'm going to be honest in this book in ways that might make you uncomfortable. I'm going to talk about my weight in real numbers. I'm going to talk about the tirzepatide I bought from a gray market peptide supplier in China because I couldn't afford the name-brand version. I'm going to talk about the time I sat in my truck at 2 AM, genuinely believing I was having a heart attack, and chose not to call 911 because I was embarrassed about how much I weighed.

I'm going to talk about these things because the fitness industry is built on curated transformation stories, and I don't have one of those. I have a messy, complicated, non-linear story that includes a lot of failure. And I think that's more useful to you than a highlight reel.

If you want perfection, this isn't your book. If you want the truth, keep reading.

Let's begin.

HOW TO USE THIS BOOK

You can read this book cover to cover, and I'd recommend that for your first pass. But it's also designed to work as a reference you come back to. Here's how I'd use it:

If you're brand new to all of this: Start with Chapter 1 (my story, so you know you're not alone), then Chapter 2 (the willpower myth, so you stop blaming yourself), then Chapter 3 (nutrition, because that's where the fastest results come from). After that, go in order.

If you've already got nutrition mostly figured out: Skip to Chapter 4 (walking) or Chapter 5 (strength). Movement is the next highest-leverage change after food.

If you're on or considering a GLP-1 medication: Read Chapter 7 first. I'm honest about the side effects, the muscle loss risk, and what you need to do to protect yourself. Then go back to Chapter 3, because protein becomes even more critical when you're on these medications.

If you're a woman over 40: Read Chapter 10. It's written with the specific hormonal challenges you're facing in mind, and it adjusts the framework accordingly.

If you just want the quick start: Go to Chapter 11 (Putting It All Together). It has a 30-day plan that sequences everything in the right order. Then go back and read the chapters that cover each phase in more detail.

Throughout the book, you'll see references to the LoadLine Tracker at loadlinefitness.com/tracker. You don't need it. A notebook works fine. But the tracker was built around this exact system, so if you want a tool that's already organized the way this book is organized, it's there and it's free.

One more thing: this book has a companion community. You can find No Excuses Jesse on Instagram and TikTok, and the LoadLine Fitness website has information about coaching programs and community access. You don't need any of that to succeed with this book. But if you

want accountability and support from people who are doing the same work, it exists.

Alright. Let's get into it.

Chapter 1: My Story

I didn't get fat overnight. That's what I told myself for years, anyway. The truth is, I got fat one bad decision at a time -- one skipped meal turned into one fast food binge, one "I'll start Monday" turned into one more month of doing nothing. One more beer on a Tuesday night because I was stressed. One more drive-through because I didn't have the energy to cook. And by the time a doctor finally looked me in the eye and said the words I'd been avoiding, I was standing on a scale that read 340 pounds.

I remember the fluorescent lights in that exam room. They hummed -- this low, electric buzz that drills into your skull when the room is quiet. And the room was quiet. The paper on the exam table crinkled every time I shifted my weight, which I did constantly because nothing about that room was comfortable for a man my size. The chair with the armrests was too narrow. The blood pressure cuff barely closed around my upper arm -- the nurse had to go get the large cuff from another room, and she did it without making a big deal of it, but I could feel the pause. The gown -- don't even get me started on the gown -- it was like wrapping a beach towel around a refrigerator.

The doctor walked in, glanced at his clipboard, and sat down on the rolling stool. He was a small man with wire-rimmed glasses and calm hands. He didn't sugarcoat it. "Your A1C is 6.4. You're one bad meal away from a Type 2 diabetes diagnosis. Your cholesterol is through the roof. Your liver enzymes are elevated. At the rate you're going, we're having a very different conversation in five years -- and you might not like the outcome."

He wasn't angry. He wasn't judgmental. He was just... honest. And that was worse. Because when someone's disappointed in you, you can get defensive. When someone's angry, you can get righteous. When someone's just laying out facts in a calm, clinical voice, you have to sit there and absorb them. There's nothing to push against. So I sat there. The fluorescent lights kept humming. The paper kept crinkling. And I felt something I hadn't felt in a long time about my body: shame.

Not the kind of shame that motivates you. Not the kind that lights a fire under your ass. The heavy kind. The kind that sits in your chest like a stone and makes you want to go home, close the blinds, and eat until you can't feel it anymore. Which is exactly what I did for the next three months.

I want to be honest about that because this book isn't about some perfect transformation story where I heard the wake-up call and immediately changed my life. I didn't. I heard the alarm, hit snooze, and pulled the covers over my head. I stopped going to follow-up appointments. I threw away the pamphlets about pre-diabetes. I ate my way through the fear the way I'd eaten my way through every other uncomfortable emotion for the past decade. That's what most people do. And if you've done it too, I need you to know: that doesn't make you broken. It makes you human.

What finally got me moving wasn't motivation. It wasn't inspiration. It was a trip to Thailand.

My buddy had been bugging me for months to come along. "It's cheap, the food's incredible, the beaches are unreal." I kept saying no. Not because I didn't want to go -- I was desperate to go -- but because I knew what traveling at 340 pounds actually looked like. It looked like airplane seatbelt extenders and the quiet humiliation of asking for one. It looked like walking two blocks in humid heat and needing to sit down. It looked like being the fattest person in every photograph, every restaurant, every temple.

I went anyway. It was all of those things.

The flight was sixteen hours of hell. My knees pressed into the seat in front of me so hard that the person reclined once, felt my bones, and sat back up for the rest of the flight without a word. I didn't sleep. My hips ached from being wedged into a space designed for someone sixty pounds lighter. I spent most of the flight standing in the back galley, letting the flight attendants step around me, pretending I was stretching when really I was just trying to breathe.

Bangkok was hot. Not warm -- hot. The kind of hot that turns your shirt into a wet towel in ten minutes. I was sweating before we left the hotel. My thighs chafed with every step. Walking through the markets, I had to stop every few minutes -- not to look at anything, but to catch my breath. I watched other tourists -- fit ones, average ones, people my age -- walking freely, climbing temple steps without a second thought, sitting in chairs with arms without worrying if they'd fit. They didn't think about their bodies. Their bodies were just vehicles that carried them through the world. Mine was a prison.

The worst moment came at a restaurant. We sat at a low table on the floor -- traditional seating. I couldn't get down. My buddy saw me struggling and casually suggested we grab a regular table in the back, like it was his idea. He was good like that. But I saw the couple at the next table glance over. They didn't mean anything by it. They probably forgot about it thirty seconds later. I didn't forget. I still haven't.

I came home from Thailand weighing 352 pounds. I'd gained twelve pounds in two weeks. The food there really was incredible, and I'd eaten all of it -- every night market stall, every plate of pad thai, every mango sticky rice. I ate like a man who'd given up on his body and was just trying to taste something good before it quit on him.

Back home, the weight kept climbing. I hit 360. Then 365. Then 368. I stopped stepping on the scale. I stopped going to the doctor. I bought bigger clothes and told myself I was "just a big guy." I said that phrase so many times it started to sound like truth. I said it to friends, to family, to myself in the mirror while I avoided making eye contact with my own reflection. It wasn't truth. It was a lie I told myself so I wouldn't have to do the hard thing.

The hard thing came anyway.

It came at 2 AM on a Tuesday night when I woke up gasping. My heart was racing -- not like after exercise, but wrong. My chest felt tight, like someone was sitting on it. My left arm tingled, this pins-and-needles feeling that wouldn't stop. I sat on the edge of my bed in the dark, alone, genuinely believing I was having a heart attack at thirty-four years old. I could feel my pulse in my temples. The bedroom felt too small.

I didn't call 911. Do you know why? Because I was embarrassed. I didn't want the paramedics to see how much I weighed. I didn't want the neighbors to see the ambulance. I didn't want to be the fat guy who had a heart attack at thirty-four. So I sat there in the dark, waiting to die, and when I didn't die, I went back to sleep. Like my life wasn't worth a phone call.

That was my rock bottom. Not the doctor's warning. Not the Thailand humiliation. Not the number on the scale. Sitting alone in the dark, too ashamed to call for help, choosing potential death over potential embarrassment. That's where "just a big guy" takes you. That's where excuses lead. Not to some dramatic cinematic moment of clarity -- to a dark room and a silent phone and a man who'd rather die quietly than ask for help.

The next morning, I didn't make a vision board. I didn't buy a gym membership. I didn't start a diet or post a "Day 1" selfie. I made one decision: I was going to stop lying to myself. I was going to look at my life exactly as it was -- no filters, no excuses, no "just a big guy" -- and I was going to fix one habit at a time.

Not all of them. Not at once. One habit. At a time.

That first habit was breakfast. I was skipping it every day, running on nothing but coffee until noon, then bingeing on whatever was fastest because my blood sugar had crashed through the floor. So I ate breakfast. That's it. That was day one. A plate of eggs and some toast at 7 AM instead of a gas station feast at noon.

It wasn't sexy. Nobody was going to clap for me. There's no before-and-after photo that captures "ate eggs instead of nothing." But it was real. And real is the only thing that has ever actually worked.

This book is about that process -- the unglamorous, unfiltered, one-habit-at-a-time process of rebuilding your metabolic health from the ground up. No magic supplements. No secret workout programs. No "lose 30 pounds in 30 days" nonsense. Just real strategies for real people who are tired of lying to themselves.

I'm not a doctor. I'm not a nutritionist. I'm a guy who was 368 pounds, sitting in the dark at 2 AM, too afraid to call an ambulance. And I figured it out. Not perfectly. Not quickly. But permanently.

You can too. But first, you have to stop making excuses.

Let's start.

Chapter 2: Why Willpower Isn't the Problem

If you've tried and failed before, here's the first thing I need you to hear: it wasn't because you don't have enough willpower. I believed that about myself for years. Every time a diet fell apart or a gym membership went unused after March, I filed it under "I just don't have the discipline." That belief kept me stuck longer than the extra weight ever did.

Here's what I know now that I didn't know then. Willpower is not a character trait you either have or don't have. It's a limited resource, and it runs out. If your whole plan depends on gritting your teeth through hunger, exhaustion, and stress every single day, you don't have a weak will. You have a plan that was never going to survive contact with real life.

What I Actually Had Wrong

Look back at what I told you in Chapter 1. I went on tirzepatide and it worked, the weight came off. Then I went to Thailand for two weeks, and it came right back. Nothing about my willpower changed on that trip. What changed is that the medication was carrying the entire load, and the moment my routine shifted, there was nothing else underneath it to hold the results in place.

That's the pattern behind almost every failed diet or program, mine included. You pull one lever (usually food, sometimes a gym plan) and ignore the other three that actually hold the whole thing together. Then something disrupts that one lever, a trip, a stressful month, a holiday, an injury, and the whole structure collapses. It looks like a willpower failure. It's actually a systems failure.

The research backs this up. Studies on self-control consistently show that willpower functions more like a muscle than a personality trait. It fatigues with use. A 2010 meta-analysis published in *Psychological Bulletin* found that acts of self-control draw on a limited pool of mental resources that can be used up. When you spend all day making difficult decisions at work, resisting snacks,

managing stress, and trying to force yourself to the gym, you're depleting the same reserve. By evening, the reserve is empty, and that's exactly when most people break their plan.

This isn't weakness. This is how human brains are wired. The answer isn't to build more willpower. It's to build systems that require less of it.

The Four Pillars

Everything in this book comes back to four things:

1. Nutrition. What you eat, and how it affects your hunger, your blood sugar, and your energy.
2. Movement. Walking and light strength work, not necessarily a gym.
3. Sleep. The pillar almost nobody treats seriously, and the one that quietly sabotages the other three when it's ignored.
4. Knowing your numbers. The bloodwork and measurements that tell you what's actually happening inside your body, instead of guessing.

Most programs, and most people, pick one of these and hope it's enough. A diet plan with no attention to sleep. A workout plan with no attention to food. A medication with no attention to any of it. When you only pull one lever, you're not building a system. You're borrowing time.

Let me give you a concrete example. I know someone, a friend, who went on a strict keto diet. Lost 40 pounds in three months. Looked great, felt great, was posting about it everywhere. Then he started sleeping five hours a night because of work stress. Within two months, the cravings hit so hard he was eating entire pizzas at midnight. The weight came back, plus some. He blamed his willpower. But his willpower didn't fail. His sleep failed, and it took his nutrition down with it.

That's a systems failure. If he'd been managing all four pillars, the stress spike wouldn't have been a death sentence for his progress. It would have been a rough week.

Why I Never Gave the Old Diets a Real Chance

I've started more diets and programs than I care to count, and looking back, most of them failed for the same two reasons. First, I never gave them real time. I wanted results in two weeks, and

when they didn't show up fast enough, I moved on to the next thing. Second, I never touched the things underneath the surface: the food addiction that had me eating every two or three hours whether I was hungry or not, the sleep apnea I ignored for years, and the simple fact that I barely moved.

You can follow a meal plan perfectly for a month and still fail long term if you never deal with why you were eating that way in the first place. You can lose weight on a medication and still end up back where you started if you never rebuild your sleep and your activity underneath it.

The Decision Fatigue Trap

Here's something nobody talks about. Most diet plans fail because they require you to make dozens of decisions every single day. What to eat, how much, when, whether this food is "allowed," whether you've "earned" a treat. Every one of those decisions costs willpower. And every time you make a decision, you're slightly more likely to fail on the next one.

The system I'm going to teach you in the following chapters is designed to remove decisions. You won't be counting calories or weighing portions. You'll be following a plate structure that makes protein and fiber the default. You won't be negotiating with yourself about whether to walk. You'll have a daily step target that's built into your routine. You won't be wondering about sleep. You'll have a wind-down time that's as automatic as brushing your teeth.

The fewer decisions you have to make, the more willpower you save for the moments that actually matter, the stressful day, the holiday meal, the week when everything goes sideways.

What This Book Is Actually Asking You to Do

I'm not going to hand you a 30-day miracle plan. What I'm going to do, chapter by chapter, is walk you through each of the four pillars the way I had to learn them, the honest version, not the version that's easy to sell. Nutrition without the diet culture nonsense. Movement that starts with walking, not a gym contract. Sleep, treated as seriously as it deserves. And your actual numbers, so you're managing your health with data instead of guesswork.

None of this requires you to be a different person than you are right now. It requires you to stop pulling one lever and hoping, and start building all four at once, even if you start small on each one. That's the difference between a result that lasts two weeks and a change that actually holds.

No Excuses

The weight didn't come off because I found more willpower. It came off because I built a system that didn't need much of it. That's what I'm going to show you how to do.

Chapter 3: Nutrition, Without the Diet Culture

I need to say something before we start this chapter, and I mean it: I am not going to put you on a diet.

I've been on enough diets to know that they don't work. Not long-term. Not for most people. The diet industry is worth over \$70 billion in the United States alone, and if diets actually worked, that number would be zero because everyone would have figured it out by now. What I'm going to give you instead is a framework, some principles you can actually live with, day after day, without white-knuckling your way through dinner or crying into a sad salad.

Here's what I believe: you can eat food that tastes good, keeps you full, and supports your metabolic health. You can do this without weighing every bite, tracking every macro, or eliminating entire food groups. You can also do this while still eating pizza sometimes. I know that sounds radical in a world that wants you to believe you need to earn your food through punishment, but it's true.

So let's talk about how.

Protein First: The Single Biggest Lever You Can Pull

If I could only give you one piece of nutritional advice for the rest of your life, it would be this: eat more protein.

I'm not talking about going full carnivore or choking down dry chicken breast until you want to cry. I'm talking about making protein the anchor of your meals, building your plate around it first, and then filling in everything else. The reason is simple: protein does things that carbohydrates and fat

don't do as well. It keeps you full longer. It preserves muscle mass, which is critical as you age and absolutely non-negotiable if you're on a GLP-1 medication where muscle loss is a real risk. It has a higher thermic effect, meaning your body actually burns more calories digesting protein than it does digesting carbs or fat. And it helps stabilize blood sugar, which means fewer energy crashes, fewer cravings, and less of that "I'm starving and I'll eat anything" feeling at 3 PM.

So how much protein do you actually need? Here's where I want to give you real numbers, not vague hand-waving.

Aim for 0.7 to 1 gram of protein per pound of your target bodyweight per day.

Not your current bodyweight, your target. If you want to weigh 180 pounds, you're aiming for roughly 125 to 180 grams of protein daily. If your target is 140 pounds, that's 100 to 140 grams. If that sounds like a lot, I hear you. Most people are eating somewhere around 50 to 70 grams a day, which isn't even close.

Here's how to think about it in real food terms:

- A 6-ounce chicken breast has about 42 grams of protein.
- A cup of Greek yogurt has about 17 to 20 grams.
- Two eggs give you about 12 grams.
- A can of tuna has about 30 grams.
- A scoop of protein powder (whey or plant-based) is typically 20 to 30 grams.
- A 6-ounce serving of salmon has about 34 grams.
- A cup of cottage cheese has about 25 grams.
- A cup of cooked lentils has about 18 grams.

So if you're targeting 150 grams a day, you need protein at every meal and probably one protein-rich snack. That's doable, it just takes some intention. A breakfast with eggs and Greek yogurt gets you 30 grams before you even leave the house. A chicken breast at lunch puts you at 70. A serving of salmon at dinner gets you to 105. Add a protein shake or some cottage cheese as a snack, and you're there.

If you're currently eating very little protein, don't try to hit your target tomorrow. Add one protein-rich food to one meal this week. Next week, add another. Build up over a few weeks. You'll notice the difference in how full you feel, how your energy holds up, and how your cravings start to quiet down.

The Plate Structure: What a Meal Actually Looks Like

Okay, so protein is the anchor. But what does a plate actually look like? Here's the simple version I use, and I want to be clear, this is a guideline, not a law.

Visualize your plate divided roughly into thirds:

- One-third protein. This is your anchor. Chicken, fish, eggs, tofu, lean ground beef, Greek yogurt, cottage cheese, beans, whatever you'll actually eat. Aim for a palm-sized portion, which is roughly 4 to 6 ounces for most people.
- One-third vegetables and/or fruit. These are your fiber sources. More on fiber in a second, but this is where the volume and micronutrients come from. Think broccoli, spinach, mixed greens, roasted peppers, berries, an apple, whatever you like.
- One-third everything else. This is your carbs and fats, rice, potatoes, bread, pasta, avocado, olive oil, cheese, nuts. This is the part that varies the most based on your preferences, your activity level, and your goals.

That's it. That's the plate. You don't need to weigh it. You don't need to measure it with cups. Just eyeball it: protein first, then veggies, then whatever else fits. If you're eating a stir-fry, load up on the chicken and vegetables and have a moderate scoop of rice. If you're having a sandwich, make it with a generous portion of turkey, pile on the lettuce and tomato, and have your chips on the side.

One more thing about the plate structure: eat your protein and vegetables first. If you fill up on those before you get to the bread basket or the pasta, you'll naturally eat less of the stuff that spikes your blood sugar and leaves you hungry again two hours later. This isn't about restriction, it's about sequencing.

Fiber: The Unsung Hero

Fiber is the thing nobody talks about because it's not sexy. You're not going to see a fitness influencer posting a "fiber check" selfie. But fiber does incredible things for your metabolic health. It slows digestion, which means your blood sugar doesn't spike as hard after a meal. It feeds the good bacteria in your gut, which affects everything from your mood to your immune system. And it keeps you full. A meal with 10 grams of fiber is going to keep you satisfied way longer than a meal with zero.

Aim for 25 to 35 grams of fiber a day. Most Americans get about 15. Here are easy ways to bump it up:

- Add berries to your breakfast. A cup of raspberries has 8 grams of fiber.
- Throw beans or lentils into your meals. Half a cup of black beans has about 7 grams.
- Eat whole fruits instead of drinking fruit juice.
- Choose whole grain bread over white bread when you can.
- Eat the skin on your apples and potatoes.
- Add a handful of vegetables to whatever you're already cooking.

You don't need a fiber supplement (though if you want one, psyllium husk is fine). Just eat more plants. That's the whole secret.

Cutting Liquid Sugar

Soda. Juice. Sweetened coffee drinks. Energy drinks. Sports drinks. Fancy lemonades.

These are, in my opinion, the single easiest place to make a change with the biggest payoff. A 20-ounce soda has about 65 grams of sugar and zero nutritional value. A large sweet tea from a fast food restaurant can have 50+ grams. A "healthy" smoothie from a chain restaurant can have 60 to 80 grams of sugar, more than a candy bar.

Liquid sugar is particularly brutal because your body doesn't register it the same way it registers solid food. You can drink 300 calories of sugar and still be hungry 30 minutes later. Your brain doesn't get the same satiety signals.

I'm not saying you can never drink anything sweet again. But if you're currently drinking your calories in the form of sugar, this is the single highest-leverage change you can make. Switch to water, sparkling water, unsweetened tea, black coffee, or coffee with a splash of cream. If you need flavor, add a squeeze of lemon or lime, or use a zero-calorie flavoring. If you drink juice, eat the fruit instead, you'll get the fiber and you'll eat less of it because chewing takes longer than sipping.

Hydration: The Thing You're Probably Not Doing Enough Of

Here's a section that wouldn't be in most nutrition books, but it should be: water.

Most people are mildly dehydrated most of the time. And when you're dehydrated, your body sends signals that feel a lot like hunger. You think you're hungry, but you're actually thirsty. You get a headache and reach for a snack instead of a glass of water. You feel sluggish and assume you need caffeine when what you need is 16 ounces of water.

The basic rule of thumb: drink at least half your bodyweight in ounces of water per day. If you weigh 200 pounds, that's 100 ounces, roughly three of those 32-ounce water bottles. If you weigh 160, that's 80 ounces.

But here's the important part, especially if you're on a GLP-1 medication (Ozempic, Wegovy, Mounjaro, Zepbound, or any of the others): you need even more. GLP-1 medications slow gastric emptying, which means everything moves through your system more slowly, including water. They also reduce your thirst cues. And if you're eating less, which most people on these medications are, you're getting less water from food. The combination means dehydration is one of the most common side effects, and it causes headaches, fatigue, constipation, and kidney stress.

If you're on a GLP-1, aim for your full bodyweight in ounces. A 200-pound person should be drinking 200 ounces of water a day. I know that sounds like a lot. It is. You'll be going to the

bathroom constantly. But it makes an enormous difference in how you feel.

Practical tips: carry a water bottle everywhere. Set a timer on your phone to drink every hour. Drink a full glass of water before every meal. If you hate plain water, add electrolytes (zero-sugar ones, Liquid IV, LMNT, Nuun, or even just a pinch of salt and a squeeze of lemon). Coffee counts toward hydration, but for every cup of coffee, drink an extra cup of water.

Reducing Ultra-Processed Food

I'm not going to tell you to never eat ultra-processed food, because that's unrealistic for most people and also because some of it tastes really good. But I am going to tell you to eat less of it, and here's why.

Ultra-processed foods, things like chips, candy bars, frozen dinners, fast food, packaged snacks, most breakfast cereals, are engineered to be hyper-palatable. They're designed in labs to hit the perfect combination of salt, sugar, and fat that makes you want to keep eating. They bypass your natural satiety signals. They're calorie-dense but nutrient-poor. And they're the single biggest driver of the metabolic health crisis we're in.

The goal isn't perfection. The goal is shifting the ratio. If 70% of what you eat right now is ultra-processed, try to get it down to 50%. Then maybe 40%. You do this by adding more whole foods, actual meat, actual vegetables, actual fruit, actual grains, not by trying to eliminate everything you enjoy.

A useful question to ask yourself: could I make this at home with ingredients I recognize? A chicken breast with rice and broccoli, yes. A Doritos Locos Taco, no. That doesn't mean you can never eat the taco. It means the taco shouldn't be your default Tuesday lunch.

Two Days of Actual Eating: A Sample Meal Plan

I know that principles are great in theory but sometimes you just want someone to tell you what to eat. So here are two sample days, real foods, real portions, no weird ingredients. These are not prescriptive. They're examples to show you what this looks like in practice.

Day One (~145g protein, ~30g fiber)

Breakfast: 3 eggs scrambled with 1 cup of spinach and 1 ounce of cheddar cheese, cooked in 1 teaspoon of olive oil. 1 slice of whole wheat toast with half an avocado. Black coffee or tea.

Protein: ~30g. Fiber: ~6g.

Lunch: Large salad with 6 ounces of grilled chicken breast, mixed greens, cherry tomatoes, cucumber, a quarter cup of chickpeas, 2 tablespoons of olive oil and vinegar dressing. 1 small apple on the side.

Protein: ~45g. Fiber: ~9g.

Snack: 1 cup of plain Greek yogurt with half cup of blueberries and 1 tablespoon of chia seeds.

Protein: ~22g. Fiber: ~6g.

Dinner: 6 ounces of salmon, baked with lemon and dill. 1 cup of roasted broccoli with garlic. Three-quarters cup of cooked brown rice. Side of mixed green salad with 1 tablespoon of olive oil dressing.

Protein: ~38g. Fiber: ~7g.

Evening (if hungry): 1 string cheese and a handful of almonds (about 15).

Protein: ~10g. Fiber: ~2g.

Day Two (~140g protein, ~33g fiber)

Breakfast: Overnight oats made with half cup rolled oats, 1 cup unsweetened almond milk, 1 scoop of vanilla protein powder, 1 tablespoon of peanut butter, and half cup of sliced strawberries. Mixed the night before, eaten cold or warmed up.

Protein: ~35g. Fiber: ~8g.

Lunch: Turkey and veggie wrap: 1 large whole wheat tortilla, 5 ounces of sliced turkey breast, lettuce, tomato, roasted red peppers, 1 tablespoon of hummus, and mustard. Side of baby carrots with 2 tablespoons of ranch dressing.

Protein: ~38g. Fiber: ~7g.

Snack: 1 cup of cottage cheese with half cup of pineapple chunks.

Protein: ~28g. Fiber: ~2g.

Dinner: 6 ounces of lean ground beef (93% lean) made into a bowl: seasoned with taco seasoning, served over 1 cup of shredded lettuce, half cup of black beans, a quarter cup of salsa, 2 tablespoons of sour cream, and a squeeze of lime. No tortilla needed, it's a bowl.

Protein: ~45g. Fiber: ~12g.

Evening (if hungry): 1 small square of dark chocolate and a cup of herbal tea.

Protein: ~1g. Fiber: ~1g.

Notice what's NOT in these meal plans: no special foods, no supplements you've never heard of, no ingredients you can only find at a health food store. This is regular grocery store food. The portions are reasonable. There's room for dark chocolate. Nobody's suffering here.

The Grocery Starter List

If you're staring at the meal plans above and thinking "I don't even know what to buy," here's a starter grocery list that covers most of what you need. Buy these once and you've got the foundation for dozens of meals:

Protein sources:

- Chicken breast (boneless, skinless)
- Eggs
- Ground beef (93% lean or leaner)
- Salmon fillets
- Turkey breast (sliced or whole)

- Canned tuna
- Plain Greek yogurt
- Cottage cheese
- String cheese
- Protein powder (whey or plant-based, whatever you prefer)
- Canned black beans
- Canned chickpeas

Vegetables and fruits:

- Mixed greens / spinach
- Broccoli
- Cherry tomatoes
- Cucumbers
- Bell peppers
- Baby carrots
- Onions
- Garlic
- Berries (strawberries, blueberries, fresh or frozen)
- Apples
- Lemons and limes
- Bananas

Carbs and starches:

- Brown rice

- Rolled oats
- Whole wheat bread
- Whole wheat tortillas
- Potatoes (sweet or russet)

Fats and cooking essentials:

- Olive oil
- Avocados
- Almonds
- Peanut butter (natural, no added sugar)
- Chia seeds

Flavor and condiments:

- Salsa
- Hummus
- Mustard
- Hot sauce
- Vinegar (balsamic or apple cider)
- Salt, pepper, garlic powder, cumin, paprika, Italian seasoning
- Lemon juice

You'll notice this isn't a "clean eating" purity list. There's cheese, sour cream, peanut butter, and bread. The point is to have real food on hand so that when you're hungry, the easy choice is also a good choice. If your fridge has chicken, broccoli, and rice in it, you'll make a decent meal. If your fridge has nothing but condiments and a half-empty bottle of ketchup, you're ordering pizza.

Food Addiction: The Elephant in the Room

I saved this section for last because it's the hardest one to talk about, and also the most important for some of you.

If you've ever eaten an entire bag of chips when you weren't hungry. If you've ever hidden food. If you've ever felt physical cravings for specific foods, sugar, fast food, salty snacks, that felt more like need than want. If you've ever eaten past the point of being full and felt unable to stop. If you've ever felt shame after eating.

You're not weak. You're not lacking willpower. You might be dealing with food addiction, and it's more common than anyone wants to admit.

The science is real. Ultra-processed foods, especially those engineered with the right combination of sugar, fat, and salt, light up the same reward pathways in the brain as addictive substances. Dopamine spikes. Tolerance builds. You need more to get the same feeling. Withdrawal happens when you stop. This isn't metaphorical. Brain imaging studies show the same patterns.

Here's what I want you to know: acknowledging this isn't about shame. It's about strategy. If you know that certain foods trigger a binge response in you, that's useful information. It doesn't mean you're broken. It means you need a plan.

Practical strategies that actually help:

1. Identify your trigger foods. Be honest with yourself. For me, it was chips. I could eat one chip and then I'd eat the entire bag. For you it might be cookies, ice cream, candy, fast food, or something else entirely. Write them down. You don't have to eliminate them forever, but you need to know what they are.
2. Don't keep trigger foods in the house. This is the single most effective thing you can do. Willpower is a finite resource, and it's weakest at 9 PM when you're tired and the kitchen is right there. If the chips aren't in your pantry, you can't eat the chips. You can still have them, at a friend's house, at a restaurant, as a deliberate treat you bought in a single-serve portion. Just don't give yourself 24/7 access.
3. Eat enough during the day. Binge eating is almost always preceded by restriction. If you eat a

tiny breakfast and a sad lunch, by 5 PM your brain is screaming for calories and it's going to reach for the most calorie-dense, fastest-absorbing thing it can find, i.e., junk food. Eat real meals with enough protein and fiber, and the evening cravings get dramatically better.

4. Use the HALT method. When a craving hits, ask yourself: Am I Hungry? Am I Angry? Am I Lonely? Am I Tired? If you're genuinely hungry, eat something, protein first. If you're angry, lonely, or tired, food isn't going to fix that. Call someone. Go for a walk. Take a nap. Journal. Do something that actually addresses what you're feeling instead of numbing it with Doritos.

5. Create friction. Make it harder to get the food you're trying to eat less of. Don't have it delivered, make yourself drive to get it. Don't buy it in bulk, buy the smallest single-serve package. The more steps between you and the food, the more chances your rational brain has to intervene.

6. Consider professional help. If your relationship with food feels genuinely out of control, regular binges, purging, severe restriction followed by overeating, eating in secret, feeling unable to stop even when you want to, please talk to someone. A therapist who specializes in eating disorders or food addiction can be life-changing. Cognitive behavioral therapy (CBT) has strong evidence for treating binge eating. Some people benefit from medication. There's no shame in asking for help with this any more than there's shame in asking for help with depression or anxiety. It's the same brain chemistry.

7. Practice self-compassion after a slip. You will eat the whole bag of chips again at some point. You will have a day where you eat nothing but garbage. When that happens, the worst thing you can do is decide you've failed and give up. The second worst thing you can do is "punish" yourself with restriction the next day, that just sets up another binge. What you do instead: you notice it happened, you don't catastrophize it, and you eat a normal meal at the next mealtime. One bad meal doesn't ruin anything. One bad day doesn't ruin a week. You just keep going.

The Honest Truth About Nutrition

Here's what I've learned after years of trying to eat better: consistency beats perfection every single time.

You don't need a perfect diet. You need a good-enough diet that you can sustain for years, not

weeks. You need to eat protein most of the time, vegetables most of the time, drink enough water, and not drink your calories in sugar. That's the 80% of it. The other 20% is everything else, macros, meal timing, supplements, "clean" eating, all of which matters much less than the food industry wants you to believe.

You're going to have days where you eat fast food. You're going to have weeks where vegetables barely happen. You're going to have moments where you eat an entire sleeve of Oreos and wonder what went wrong. That's being human. That's not failure. Failure is letting those moments convince you that the whole project is pointless and going back to eating whatever you want, whenever you want, in whatever quantity you want, with no attention to how it makes you feel.

So eat the protein. Eat the vegetables. Drink the water. Buy real food when you can. And when you eat the pizza, enjoy the pizza, and then eat a normal breakfast tomorrow. That's it. That's the whole system.

No diet required.

Chapter 4: Just Get Up and Walk

I told you in Chapter 1 that I couldn't walk for two or three minutes without needing to stop and rest. I want to sit with that for a second, because it's easy to read past a sentence like that. That wasn't a bad day or a rough patch. That was my baseline. Walking to the mailbox and back was a task that required recovery time.

So when people ask me what the single most underrated tool in this whole process is, I don't hesitate. It's walking. Not a gym program, not a personal trainer, not a fitness tracker with a hundred metrics. Just walking, done consistently, is one of the most effective things you can do for your metabolic health, and it's the thing almost everyone overlooks because it feels too simple to matter.

Why Walking Works

When you walk, especially after a meal, your muscles pull glucose out of your bloodstream to use as fuel, without needing as much help from insulin. Over time, that adds up to real improvements in how your body handles blood sugar. A study published in *Diabetes Care* found that just 15 minutes of walking after each meal was more effective at managing blood sugar levels in older adults than a single 45-minute walk at another time of day. The timing matters. The post-meal walk isn't just exercise, it's a direct intervention in how your body processes food.

Walking is also low impact, which means it doesn't beat up your joints the way running or high intensity training can, especially if you're carrying extra weight the way I was. And it's something you can do literally anywhere, no equipment, no membership, no excuse.

There's another benefit that rarely gets mentioned: walking is one of the most effective stress-reduction tools available. Research from Stanford University showed that walking increased creative output by an average of 60 percent. It lowers cortisol, the stress hormone that drives belly fat storage. It improves sleep quality when done earlier in the day. And it does all of

this without requiring you to think about form, technique, or recovery the way strength training does.

Start Wherever You Actually Are

If you're anywhere close to where I started, don't set a goal of 10,000 steps on day one. That number gets thrown around everywhere, but it means nothing if you're currently doing 2,000 steps and every walk ends with you needing to sit down. Find your actual starting point first. Then add 1,000 to 1,500 steps a week until you build up toward 7,000 to 10,000 steps a day. Slow progress you can sustain beats an ambitious target you abandon in a week.

Here's what that actually looks like for someone starting from where I was:

Week 1-2: Two to three short walks per day, 5 to 10 minutes each. Aim for 3,000 to 4,000 steps total. If that's too much, start with one walk and build from there.

Week 3-4: Add one additional 10-minute walk. Push toward 5,000 to 6,000 steps.

Week 5-6: Extend one walk to 20 minutes. Target 7,000 to 8,000 steps.

Week 7-8: Add a fourth walk if needed, or extend existing walks. Push toward 9,000 to 10,000 steps.

That's two months of building. Not two days. Not two weeks. Two months. And by the end of it, you'll have a capacity you didn't have before, built slowly enough that it actually holds.

The Post-Meal Walk

If you take nothing else from this chapter, take this. A 10 to 15 minute walk within 30 to 60 minutes of your biggest meal can meaningfully reduce the blood sugar spike that follows eating. This is one of the simplest, highest-leverage habits in this entire book, and it costs you nothing but a few minutes.

I wish someone had told me this years ago, back when I was eating every two or three hours and never moving in between. A short walk after a meal wouldn't have fixed everything on its own, but it would have been one more lever working in my favor instead of against me.

The research here is consistent and strong. A 2022 meta-analysis in Sports Medicine analyzed 7 studies and found that even light walking after meals significantly improved blood glucose levels compared to sitting. The effect was largest after dinner, which is typically the meal with the most carbohydrates and the one followed by the longest period of inactivity.

Make It Frictionless

The habits that stick are the ones that require the least willpower to maintain. A few ways to make walking part of your day without having to think hard about it:

Take phone calls standing up or walking instead of sitting down. This one change alone can add 2,000 to 3,000 steps a day for people who take a lot of calls.

Park farther away on purpose, every time. Not just sometimes. Every time.

Take one short walking break for every 90 minutes you spend at a desk. Set a timer if you have to.

Treat your daily walk like brushing your teeth, something you just do, not something you negotiate with yourself about.

If you have a dog, walk it. If you don't, walk like you do. The routine matters more than the reason.

The Step Count Targets That Actually Matter

There's been a lot of debate about whether 10,000 steps is a real target or just a marketing number. Here's what the research actually shows:

4,000 to 5,000 steps per day: This is the range where mortality risk starts to meaningfully decrease compared to sedentary levels. If you're currently below 3,000, getting to this range is a genuine win.

7,000 to 8,000 steps per day: A 2021 study in JAMA found that women who averaged 7,500 steps per day had significantly lower mortality rates than those at lower levels. Beyond this point, the benefits continued but at a diminishing rate.

10,000+ steps per day: Still beneficial, but the incremental gain above 8,000 is smaller than the gain from 4,000 to 7,000. Don't let the 10,000 number intimidate you out of starting.

The point is this: every step you add matters. You don't need to hit 10,000 to get real health benefits. You need to move significantly more than you are now, and keep doing it.

What This Looked Like for Me

I didn't start with 10,000 steps. I started with what I could actually do, which some days was still just a few minutes at a time. What changed wasn't some sudden burst of endurance, it was consistency, showing up for the short walk even on the days I didn't feel like it, and letting the capacity build slowly instead of forcing it.

That's the part nobody wants to hear, because it's not exciting. There's no dramatic before and after in a single walk. But stack enough of those unremarkable walks together, and they become one of the pillars holding the rest of this up.

As I write this, I average between 8,000 and 10,000 steps a day. A year ago, I could barely manage 2,000. The difference isn't motivation. It's a system that built capacity one week at a time, without asking me to be a hero on any given day.

The Science Behind the Walk

I want to give you the actual research, because when you understand why walking works at a cellular level, it stops being "just walking" and starts being medicine.

A 2022 meta-analysis published in Sports Medicine looked at the effects of interrupting prolonged sitting with light-intensity physical activity on postprandial glucose, that's the blood sugar spike you get after eating. The findings were clear: even short bouts of light activity, as little as two to five minutes of walking, significantly reduced blood sugar levels after meals compared to sitting still.

A landmark 2013 study from Indiana University found that three 15-minute walks after meals were

more effective at controlling 24-hour blood sugar levels than a single 45-minute walk at any other time of day. The timing matters. Walking after eating directly blunts the glucose spike that would otherwise trigger an insulin response, and over time, that repeated blunting improves your overall insulin sensitivity.

The step count research is equally compelling. A 2020 study in JAMA involving over 4,800 adults found that those who took 8,000 steps per day had a 51% lower risk of all-cause mortality compared to those who took 4,000 steps. And a 2021 meta-analysis published in The Lancet Public Health, analyzing data from over 15 international cohorts and nearly 50,000 people, confirmed that mortality risk decreases progressively with higher step counts, up to about 10,000 steps per day for adults under 60.

But here's the part that surprised me most: a Stanford University study found that walking increased creative output by an average of 60%. Sixty percent. The researchers called it "walking thinking." I don't know about you, but some of my best decisions about my health, my business, and my life have happened on walks. There's something about the bilateral movement, left foot, right foot, left foot, right foot, that quiets the anxious noise in your head and lets clearer thinking emerge.

What If I Can't Walk for 10 Minutes?

If you're reading this and thinking "I can't walk for 10 minutes, I can barely walk for 2," I hear you. I was there. When I started, walking to the end of my street and back was a legitimate workout. I was breathing hard. My legs ached. I had to stop once. That was my Day 1.

Here's what I want you to do: walk for whatever duration you can manage, even if it's 60 seconds, and then stop. Sit down if you need to. Rest. And then, if you can, walk for another 60 seconds. That's your first session. Two minutes total, with a break in between.

Tomorrow, do it again. And the day after that. Within two weeks, you'll be surprised at how much further you can go. The human body adapts to walking faster than almost any other form of exercise, because it's what we evolved to do. We're literally built for it.

Don't let the gap between where you are and where this book says you should be stop you from

starting. The 10,000-step target is a destination, not a prerequisite. Start with what you can do today. That's always enough.

The Post-Meal Protocol

Here's the specific protocol I follow and recommend:

Within 30 minutes of finishing a meal, take a 10 to 15-minute walk. The pace should be conversational, you should be able to talk without gasping. This isn't power walking or jogging. It's a stroll. A mosey. A gentle meander.

If the weather is bad, walk around your house. Walk up and down your hallway. Walk around your office building. Walk in place if you have to. The movement matters more than the location.

If you're at a restaurant and can't walk immediately, walk when you get to your car. Walk around the parking lot once. It takes three minutes. Those three minutes after a restaurant meal, which is typically higher in carbs and calories than a home-cooked meal, make a measurable difference in how your body handles that glucose load.

I do this after every meal, every day. It's as automatic as brushing my teeth. And the cumulative effect, over weeks and months, is transformative. My post-meal blood sugar readings, when I was tracking them with a continuous glucose monitor, were dramatically lower on days I walked after meals versus days I didn't. The data doesn't lie.

Making It Stick

The hardest part of walking isn't the walking itself. It's making it automatic. Here's what worked for me:

Attach it to something you already do. I walk after breakfast, after lunch, and after dinner. The meal is the trigger. I finish eating, I put on my shoes, I walk. No decision required. No willpower expended. It's just what happens after a meal.

Track it. I use a cheap fitness watch that counts my steps. Seeing the number go up throughout the day is motivating in a way I didn't expect. There's something satisfying about hitting your target, even if it's an arbitrary number. The LoadLine Tracker also tracks steps if you want it integrated with your other metrics.

Make it enjoyable. I don't listen to podcasts or music on most of my walks. I use the time to think, to decompress, to notice things around my neighborhood I'd never notice from my truck. But if you need a podcast to get out the door, listen to a podcast. If you need a walking buddy, find a walking buddy. The best exercise is the one you actually do.

Don't break the chain. Once you've walked every day for two weeks, the streak itself becomes motivating. You don't want to break it. You'll walk in the rain rather than lose the streak. Use that psychological momentum. It's free and it works.

Chapter 5: Strength Without the Gym

Here's something nobody tells you when you start caring about your metabolic health: muscle is the organ of longevity. Not your heart, though that matters. Not your brain, though that matters too. Your skeletal muscle. It's the largest organ in your body, it's the primary site for glucose disposal, and after about age thirty, you're losing it unless you're actively fighting to keep it.

I didn't know any of that when I started. I just knew I felt weak, tired, and soft. I also knew I wasn't going to a gym. Not because gyms are bad, they're fine, but because I had a history of buying memberships, going three times, and then avoiding the place like it owed me money. The commute, the crowds, the feeling of not knowing what I was doing in front of people who clearly did. It was enough to stop me before I started.

So I decided to build strength at home, with almost nothing. No squat rack. No cable machine. Just my body, a couple of resistance bands, and a floor. And I'm going to walk you through exactly what I did, because if I waited until conditions were perfect, I'd still be sitting on my couch reading about fitness instead of doing it.

Why Muscle Matters More Than You Think

Let me be blunt about the biology, because understanding this changed how I thought about exercise entirely.

Muscle tissue is metabolically active. Every pound of muscle you carry burns calories at rest, not a lot, but it adds up. More importantly, muscle is where your body handles glucose. When you eat carbohydrates, insulin signals your muscle cells to pull sugar out of your bloodstream. The more muscle you have, and the more insulin-sensitive that muscle is, the better your body handles the food you eat. Less circulating insulin. Less fat storage. Lower blood sugar. Better energy.

Without adequate muscle, you're essentially asking a small workforce to do a large workforce's job. Your remaining muscle cells get overtaxed, become insulin resistant, and your metabolic health deteriorates. This is why skinny people can still be metabolically unhealthy. Weight on the scale tells you almost nothing about what's happening inside.

I'm not saying you need to look like a bodybuilder. I'm saying you need enough muscle to do your job: processing food efficiently, maintaining bone density, staying functional as you age. The research is unambiguous on this. Resistance training, any resistance training, is the single most important form of exercise for long-term metabolic health.

Mobility vs. Flexibility: They're Not the Same Thing

Before we get into the exercises, I need to clear up something that confused me for months.

Flexibility is how far a muscle can passively stretch. Think of someone pushing your leg up while you lie there. Mobility is how far you can actively move a joint under control. Think of lifting your own leg up.

You can be flexible without being mobile, a dancer who can do the splits but can't hold their leg up at hip height without support. You can also be mobile without being particularly flexible. What matters for strength training is mobility: can you get into the positions you need to get into, and can you control yourself there?

I spent weeks doing static stretches before my workouts because I thought that was the warm-up. It's not. Static stretching before strength work can actually reduce your force production. What you want is dynamic movement: leg swings, arm circles, bodyweight squats to warm depth, hip circles. Save the static stretching for after your workout or as a separate session.

A good warm-up takes five minutes. Rotate your arms forward and backward. Do ten bodyweight squats. Swing each leg forward and back, side to side. Do some gentle spinal rotations. You're not trying to improve your flexibility in this moment, you're preparing your joints and nervous system for the work ahead.

The Four-Week Progression Plan

Here's the framework I followed. It's simple on purpose. Complexity is the enemy of consistency, and consistency is the only thing that actually works.

Week 1: Bodyweight Only

This week is about learning the movements and building the habit. No equipment needed. You'll do three sessions, each about twenty minutes. Rest at least one day between sessions.

The circuit:

- Bodyweight Squats: 2 sets of 10 reps. Stand with feet shoulder-width apart. Sit back like you're reaching for a chair that's slightly too far behind you. Keep your chest up, imagine someone's reading the text on your shirt. Drive through your whole foot to stand. If your heels come up, you're going too deep for now. That's fine. Go as deep as you can with flat feet.
- Push-Ups (or Modified Push-Ups): 2 sets of as many as you can with good form. If you can't do a full push-up from the floor, do them with your hands on a counter, a sturdy table, or a wall. The higher the surface, the easier it is. There is zero shame in wall push-ups. I started there. Keep your body in a straight line from head to heels (or head to knees if modified). Lower until your chest is close to the surface, then push back up. If your hips sag or pike up, you've lost the position, reset.
- Glute Bridges: 2 sets of 12 reps. Lie on your back, knees bent, feet flat on the floor. Push through your heels and squeeze your glutes to lift your hips until your body forms a straight line from shoulders to knees. Hold for one second at the top. Lower slowly. This looks easy. Done right, with a full glute squeeze at the top, it is not.
- Dead Hangs (if you have a pull-up bar or sturdy doorframe bar): 2 sets, hold as long as you can. If you don't have a bar, skip this for now, we'll come back to grip strength shortly.
- Plank: 2 sets, hold as long as you can with good form. Elbows under shoulders, body straight, glutes squeezed. When your hips start to sag, stop. A thirty-second honest plank beats a two-minute sloppy one.

Rest 60-90 seconds between sets. Don't rush. The point this week is movement quality, not intensity.

Week 2: Add a Light Resistance Band

If Week 1 felt manageable, meaning you completed all the sets and your form held up, you're ready to add resistance. Buy a set of looped resistance bands (the flat, wide ones). Get a light or medium one to start. They cost ten to fifteen dollars for a set.

- Banded Squats: Place the band just above your knees. 2 sets of 12 reps. The band will try to pull your knees inward, fight it. Push your knees out over your toes. This teaches you to activate your glutes properly in the squat, which is something most beginners struggle with.

- Push-Ups: Same as Week 1, but try to do one more rep per set than last week, or drop the surface height slightly (counter instead of wall, for example).

- Banded Pull-Aparts: 2 sets of 15 reps. Hold the band in front of you at shoulder height with arms straight. Pull the band apart by squeezing your shoulder blades together. This strengthens your upper back and rear deltoids, the muscles that counteract all the sitting and hunching we do. Keep your ribs down and your neck long.

- Banded Glute Bridges: Same as before, but with the band above your knees. 2 sets of 12 reps. You'll feel this significantly more because the band forces your glutes to work harder to keep your knees from caving.

- Plank: 2 sets, hold for time. Try to beat your Week 1 times by five to ten seconds.

Week 3: Add Sets

You're going from two sets to three on each exercise. This is where the real training stimulus starts to build. The extra set means more total volume, which means more signal to your muscles to adapt and grow.

- Banded Squats: 3 sets of 12 reps.

- Push-Ups: 3 sets of whatever rep range you can manage with good form.

- Banded Pull-Aparts: 3 sets of 15 reps.
- Banded Glute Bridges: 3 sets of 12 reps.
- Plank: 3 sets, hold for time.

If three sets feels like too much, stay at two for another week. There is no deadline. The plan is a guideline, not a law.

Week 4: Increase Reps or Slow the Tempo

This is where you choose your progression. Either do more reps (15 instead of 12) or, my preferred method, slow down the movement. Take three full seconds to lower yourself into a squat, pause for one second at the bottom, and stand back up. This "tempo" approach increases time under tension, which is a powerful driver of muscle adaptation. It also forces you to own every inch of the movement, no bouncing, no momentum, no cheating.

- Tempo Squats: 3 sets of 10 reps with a 3-second lowering phase.
- Tempo Push-Ups: 3 sets with a 3-second lowering phase.
- Banded Pull-Aparts: 3 sets of 20 reps.
- Banded Glute Bridges: 3 sets of 15 reps with a 2-second hold at the top.
- Plank: 3 sets. If you're past 45 seconds, try lifting one foot an inch off the ground for five seconds at a time, alternating legs. This is called a plank with hip extension, and it's significantly harder.

After Week 4, you repeat the cycle: go back to the rep/set scheme of Week 2 or 3, but with a heavier band. That's progressive overload, the fundamental principle of getting stronger. Do more over time, in whatever small increments you can manage.

Grip Strength: The Metric Nobody Talks About

I'm going to tell you something that changed how I think about fitness entirely: your grip strength is one of the strongest predictors of all-cause mortality that we have. Stronger grip, longer life.

This isn't a fringe finding, it's been replicated across dozens of studies involving hundreds of thousands of people.

Why? Because grip strength is a proxy for overall strength. If your grip is weak, the rest of you is probably weak too. It also correlates with bone density, cardiovascular health, and cognitive function. Researchers think it's a good marker because it reflects cumulative neuromuscular health, the kind that builds over years of physical work and activity.

The dead hang I mentioned in Week 1 is your starting point. Hang from a bar with both hands for as long as you can. Most beginners can manage ten to twenty seconds. Work up to sixty seconds over several weeks. Once you're there, try single-arm hangs, or hang while gently rotating your body.

Other grip builders: carry heavy grocery bags instead of using a cart. Squeeze a tennis ball while you watch TV. If you have a pair of heavy dumbbells or even a couple of gallon water jugs, just hold them at your sides for thirty seconds at a time. Farmer's carries are one of the best functional exercises in existence, and they require zero skill, just pick up something heavy and walk.

When I started training my grip deliberately, I noticed something unexpected: my other lifts got better. My push-ups felt more stable. My rows felt stronger. Grip is a bottleneck you don't know you have until you address it.

When to Upgrade Bands (or Move to a Gym)

You'll know it's time to move up in band resistance when you can complete all the prescribed sets and reps with good form and the last two or three reps don't feel particularly challenging. If you're finishing your sets and thinking "I could do five more easily," the band is too light. Move up.

A set of bands typically includes light, medium, heavy, and extra-heavy. You might spend two to four weeks on each level. Some people, particularly those with more muscle mass to start, will progress faster. Others will be on the medium band for two months. Both are fine.

Here's when you might consider a gym: when you've outgrown your heaviest band and you want to keep progressing. Bands have a ceiling. They provide the most resistance at the end range of

motion, which means they're great for some exercises and limited for others. At some point, if you want to keep building strength, particularly in movements like squats, deadlifts, and rows, you'll need barbells and dumbbells.

But here's the thing: you might never reach that point, and that's completely valid. A strong, mobile, metabolically healthy person who trains with bands three times a week is in a better position than ninety percent of the population. Don't let the perfect be the enemy of the done.

Common Mistakes Beginners Make

I made most of these. You probably will too. Knowing about them ahead of time doesn't guarantee you'll avoid them, but it might shorten the learning curve.

Mistake 1: Going too hard, too fast. The first week, you feel motivated, so you do five sets of everything and add exercises you saw on Instagram. Two days later, you can't sit down without using your hands. You skip the next session, then the one after that. Start with less than you think you need. You can always do more next time. You can't undo a week of being too sore to function.

Mistake 2: Skipping the warm-up. "I'll just get right into it." Five minutes of movement prep prevents injuries and improves your performance in the actual sets. It's not optional. I pulled a muscle in my shoulder doing cold push-ups once. It took three weeks to fully heal. Don't be me.

Mistake 3: Counting reps instead of focusing on quality. Ten bad squats teach your body bad movement patterns. Five good squats build strength and reinforce proper mechanics. If your form breaks down, stop the set. Ego has no place in a living room workout.

Mistake 4: Not resting enough between sets. Your muscles need time to recover between sets to perform well on the next one. Sixty to ninety seconds is the minimum. If you're breathing hard and your heart is pounding, wait longer. This isn't cardio, it's strength training. The rest is part of the workout.

Mistake 5: Expecting visible results in two weeks. Your nervous system adapts first, you get stronger before you look different. Strength gains in the first four to six weeks are almost entirely

neurological. Your brain gets better at recruiting the muscle you already have. Visible changes typically start around week six to eight, depending on your starting point and nutrition. If you quit in week three because you don't look different yet, you're quitting right before the adaptation kicks in.

Mistake 6: Ignoring what you eat. You can't out-train a bad diet, and you definitely can't build muscle without adequate protein. You don't need to count macros or buy supplements. You need to eat enough protein at each meal, roughly a palm-sized portion of meat, fish, eggs, or legumes, and you need to eat enough total food to support recovery. Training breaks muscle down. Food builds it back up. Both are required.

The Minimum Effective Dose

I want to leave you with this: three sessions a week, twenty to thirty minutes each, with a pair of resistance bands and a floor, is enough to build meaningful strength. Not Instagram-model strength. Not competitive-powerlifter strength. Enough strength to improve your insulin sensitivity, maintain your muscle mass, protect your bones, and keep you functional for decades.

That's not a consolation prize. That's the whole point.

You don't need a gym membership. You don't need a personal trainer. You don't need perfect genetics or a lot of free time. You need a plan, some consistency, and the willingness to be bad at something for a while before you get decent at it.

I was bad at this for months. My push-ups were shaky, my squats were shallow, and I had to take breaks in the middle of planks. But I kept showing up. Three times a week, every week, no matter how I felt. And slowly, almost imperceptibly, I got stronger. Not just physically, though that happened too. I got stronger in the way that matters: I became someone who does hard things even when nobody's watching and the conditions aren't perfect.

That's the real workout. The muscle is just a bonus.

Chapter 6: Sleep Is the Multiplier

Of everything the doctor told me that day in the fall of 2025, the sleep apnea diagnosis is the one I brushed off the hardest. Diabetic and high blood pressure sounded serious. Sleep apnea sounded like something you dealt with using a machine at night and otherwise ignored. I was wrong about that, and it took me a while to understand why.

Sleep isn't a separate category from the rest of your health. It's the multiplier sitting underneath everything else in this book. Poor sleep directly worsens how your body handles insulin, and it drives cravings for sugar and refined carbs the very next day, the exact foods that undo the nutrition work from Chapter 3. You can eat well, walk every day, and lift bands three times a week, and still stall out completely if your sleep is broken.

The Science of Why Sleep Matters So Much

Let me put some numbers on this so you understand why I call sleep a multiplier and not just another habit.

A study published in the *Annals of Internal Medicine* put subjects on identical calorie-restricted diets. One group slept 8.5 hours per night. The other slept 5.5 hours. Both groups lost the same amount of weight. But here's the critical difference: the sleep-deprived group lost 60 percent more muscle and 55 percent less fat than the well-rested group. Same calories. Same deficit. Dramatically different body composition.

That study changed how I thought about everything. If you're on a GLP-1 medication and losing weight quickly, but you're sleeping five hours a night, you're likely losing far more muscle than you need to. The medication is doing its job on the scale, but your body composition is getting worse, not better. Sleep is what protects the muscle while the fat comes off.

Beyond body composition, sleep directly controls two hormones that drive hunger: ghrelin and

leptin. Ghrelin tells you you're hungry. Leptin tells you you're full. When you're sleep-deprived, ghrelin goes up and leptin goes down. Your body is literally sending you stronger hunger signals and weaker fullness signals. This isn't a willpower problem. It's a biochemistry problem, and the fix is sleep, not more discipline.

Why This Gets Ignored

Nutrition and exercise are visible. You can see a plate of food or feel a workout. Sleep happens while you're unconscious, so it's easy to treat it as passive, something that just happens to you rather than something you actively manage. That was exactly my mistake. I treated my sleep apnea as a diagnosis to live with instead of a problem connected to everything else I was trying to fix.

There's also a cultural issue. In America especially, sleep deprivation is worn as a badge of honor. "I'll sleep when I'm dead" is the kind of thing people say like it's admirable. The research says the opposite: chronic sleep deprivation accelerates the path to exactly the kind of metabolic disease that nearly killed me.

The Four Levers That Actually Matter

You don't need a complicated sleep routine with a dozen rules. Focus on these four, and most of the benefit follows.

Consistent wake time. Even on weekends. This does more to anchor your body's internal clock than what time you go to bed. A wake time that swings by two or three hours from one day to the next confuses the same systems that regulate hunger and blood sugar. Pick a wake time and hold it within a 30-minute window every day, including weekends. This is the single most important sleep habit you can build.

Morning light. Five to ten minutes of natural light within an hour of waking helps set that internal clock for the day. Step outside, face the sun (don't stare at it), and let your eyes register the brightness. If you live somewhere with limited morning sun, a 10,000 lux light therapy lamp placed on your desk for 20 to 30 minutes works as a substitute. This is one of the simplest, most overlooked tools in the entire book.

Caffeine cutoff. Nothing after early afternoon. Caffeine has a half-life of five to six hours, which

means if you drink a coffee at 3 PM, half of that caffeine is still in your system at 9 PM. It quietly fragments sleep even when you don't notice yourself lying awake because of it. My cutoff is 1 PM. Find yours and hold it.

Wind-down window. Thirty to sixty minutes of dim light and no screens before bed. Keep the room cool, somewhere around 65 to 68 degrees works well for most people, and dark. Blackout curtains or a sleep mask, earplugs if you need them. This isn't luxury, it's infrastructure.

Aim for consistency, not perfection. Seven to nine hours most nights beats chasing one flawless night here and there.

What I'd Tell Someone With Sleep Apnea

If you've been diagnosed with sleep apnea, or you suspect you have it, this isn't a chapter you can substitute good habits for medical treatment. Get it evaluated and treated properly, a machine, a mouthguard, weight loss under guidance, whatever your doctor actually recommends for your case.

What I'm describing here works alongside that treatment, not instead of it. I ignored the seriousness of my own diagnosis for too long, and I don't want you making the same mistake. My CPAP machine wasn't the most glamorous piece of equipment I've ever owned, but the difference it made in my energy, my cravings, and my ability to stick with the rest of this system was enormous.

If you suspect you have sleep apnea but haven't been tested, here are the signs: loud snoring, gasping or choking during sleep, morning headaches, excessive daytime sleepiness even after what should be a full night, and difficulty concentrating. A home sleep study is inexpensive and easy, and the results can change your life.

The Nap Question

People ask me about naps. My answer: if you need a nap because you slept poorly, take a short one, 20 minutes, no more. Set an alarm. But don't use naps to compensate for a chronically broken sleep schedule. Fix the schedule first. A nap is a band-aid. A consistent sleep routine is the cure.

Where This Fits With Everything Else

Think back to Chapter 2 and the four pillars. Sleep is the one most people leave out entirely, and it's often the reason the other three pillars stop producing results even when you're doing everything else right. Fix your nutrition, keep walking, keep up the strength work, and give your sleep the same seriousness you're giving the rest of it. That's when the whole system actually starts compounding instead of fighting itself.

What If You Can't Fall Asleep?

I get this question a lot, and I dealt with it myself for years. You do everything right, you dim the lights, you put the phone away, you get in bed at a reasonable hour, and then you lie there staring at the ceiling while your brain runs through every mistake you've ever made and every problem you haven't solved.

Here's what helped me:

The 20-minute rule. If you're not asleep within 20 minutes, get up. Don't lie there fighting it. Go to another room, keep the lights dim, and do something boring until you feel drowsy. Read a physical book (not on a screen). Fold laundry. Sit in a chair and breathe. Then go back to bed. The goal is to break the association between your bed and wakefulness.

Write it down. I keep a notepad on my nightstand. When my brain won't shut up, I write down whatever's on it. Tomorrow's to-do list. The thing I'm worried about. The idea I'm afraid I'll forget. Getting it out of my head and onto paper seems to tell my brain it's safe to let go. It doesn't work every time, but it works often enough to be worth doing.

Temperature matters more than you think. Your core body temperature needs to drop about 2 to 3 degrees Fahrenheit to initiate sleep. This is why a cool room (65 to 68 degrees) helps. It's also why a hot shower 60 to 90 minutes before bed can help, the rapid cooling afterward mimics the natural temperature drop. I started sleeping with the thermostat at 66 and noticed a real difference within the first week.

Don't nap after 2 PM. If you're tired during the day, a short nap (10 to 20 minutes) before 2 PM is fine and can actually improve your alertness. But napping later in the day, or napping for longer than 20 minutes, steals from your nighttime sleep drive. Your body builds up adenosine (a sleep-promoting chemical) throughout the day. Napping clears it, and if you clear it too late, you won't have enough built up by bedtime.

Alcohol is a liar. I know, I know. A glass of wine or a beer before bed makes you feel drowsy. But alcohol fragments your sleep architecture. You fall asleep faster but spend less time in deep sleep and REM sleep, which are the restorative stages. You wake up feeling like you slept but your body didn't actually recover. If you drink, try to stop at least 3 hours before bed. Your sleep quality will improve noticeably.

Chapter 7: Supplements, Medications, and Peptides, Done Right

Let me be blunt: the supplement industry is mostly garbage. It's a multi-billion-dollar machine designed to sell hope in a bottle to people who'd rather swallow a pill than fix their diet. I know because I was one of those people. For years, I bought into the idea that the right combination of capsules could undo the damage of bad sleep, processed food, and zero movement. It can't.

But here's the other side of that coin, once you've actually done the foundational work (eating real food, sleeping enough, moving your body, managing stress), certain supplements genuinely help. Not because they're magic, but because modern life creates gaps that even a good diet sometimes can't fill. And in some cases, medications and peptides can be powerful tools when used responsibly under medical supervision.

This chapter is about separating what works from what's waste. No hype. No affiliate links. Just what I actually take, why, and what I've learned the hard way.

The Supplements I Actually Take

Creatine Monohydrate

This is the most studied supplement in sports science, and the verdict is clear: it works. Creatine helps your muscles produce energy during high-intensity exercise. It supports strength, power output, and, this is the part most people miss, cognitive function. There's growing research showing creatine benefits brain health, especially as we age.

Dosage: 3 to 5 grams per day, every day. No loading phase necessary (that old protocol of 20g/day for a week just causes bloating and wastes product). Mix it in water, coffee, a shake, doesn't matter. Timing doesn't matter either. Consistency does.

What to look for on the label: Creatine monohydrate. That's it. Not "creatine HCL," not "buffered creatine," not some fancy proprietary blend. Plain monohydrate, ideally Creapure branded (the gold standard for purity). If a product tries to sell you on a "advanced" form of creatine for three times the price, put it back.

Vitamin D3

If you work indoors, live above the 37th parallel, or don't spend significant time shirtless in midday sun, you're probably deficient. I was. My bloodwork came back at 22 ng/mL, the functional optimal range is 40 to 60 ng/mL. Vitamin D isn't really a vitamin; it's a hormone precursor that affects everything from immune function to mood to testosterone production.

Dosage: 2,000 to 5,000 IU per day depending on your blood levels. Get tested first. Take it with a fat-containing meal since it's fat-soluble. Pair it with Vitamin K2 (100 to 200 mcg MK-7 form) to direct calcium into bones instead of arteries.

What to look for: D3 (cholecalciferol), not D2 (ergocalciferol). D2 is cheaper and less effective. Softgel or liquid drops suspended in oil (like olive oil or MCT oil) absorb better than tablets.

Magnesium

Another one most people are deficient in, largely because modern agriculture has depleted our soil. Magnesium is involved in over 300 enzymatic reactions in your body, muscle contraction, nerve signaling, blood sugar regulation, sleep quality. If you get muscle cramps, feel anxious for no reason, or can't fall asleep despite being tired, low magnesium is a strong suspect.

Dosage: 200 to 400 mg per day of elemental magnesium. The form matters enormously here. Magnesium glycinate is best for sleep and relaxation (take it before bed). Magnesium citrate helps with regularity. Magnesium threonate crosses the blood-brain barrier and may support cognitive function, but it's expensive. Avoid magnesium oxide, it's the cheapest form with the worst absorption. It's basically a laxative.

What to look for: The specific form (glycinate, citrate, threonate) listed clearly on the label with the elemental magnesium amount. If the label says "Magnesium 500mg" but the fine print shows "as magnesium oxide," most of that 500mg is passing right through you.

Fish Oil (Omega-3 Fatty Acids)

Omega-3s reduce inflammation, support cardiovascular health, and improve the ratio of omega-3 to omega-6 in your body (which, if you eat a standard Western diet, is way out of whack). I take it because my diet isn't perfect, and I don't eat fatty fish every day.

Dosage: 1 to 2 grams of combined EPA and DHA per day. Note: this is NOT the same as 1 to 2 grams of "fish oil." A 1,000mg fish oil capsule might only contain 300mg of actual EPA+DHA. Read the back of the label.

What to look for: Triglyceride form (not ethyl ester) for better absorption. Check the EPA and DHA numbers on the back, not the front. Third-party tested for heavy metals and oxidation. If your fish oil smells rancid or gives you fishy burps, it's oxidized, throw it out and buy a better brand. Look for IFOS (International Fish Oil Standards) certification.

Fiber

This one isn't glamorous, but it might be the single most impactful supplement for metabolic health. Fiber slows glucose absorption, feeds your gut microbiome, improves satiety, and helps regulate cholesterol. Most people get 10 to 15 grams a day. We should be getting 30 to 50 grams.

Dosage: Start with 5 grams of supplemental fiber per day and work up to 10 to 15 grams. Go slow or you'll be painfully bloated. I use psyllium husk, it's cheap, effective, and well-studied. Take it with a full glass of water.

What to look for: Plain psyllium husk or husk powder without added sugars or artificial flavors. Whole psyllium husk has more bulk; powder mixes easier. Both work.

Protein Powder

Not a magic muscle builder. It's just convenient food. If you're hitting 0.7 to 1 gram of protein per pound of bodyweight through whole foods, you don't need it. Most people aren't, so a shake helps

close the gap.

Dosage: 20 to 40 grams per serving, whenever you need to hit your protein target. Post-workout is fine but not magical. Anytime is fine.

What to look for: Whey isolate or whey concentrate (if you tolerate dairy). For plant-based, look for a blend (pea + rice protein gives a complete amino acid profile). Check that the protein-per-serving number is close to the total scoop size. If the scoop is 40 grams but protein is only 18 grams, the rest is fillers and garbage. Avoid anything with a "proprietary protein blend," that's code for "we won't tell you how much of the cheap stuff we put in."

What NOT to Waste Your Money On

The supplement industry spends more on marketing than on research. Here's what I've learned to avoid:

Testosterone boosters. Tribulus terrestris, ashwagandha "testosterone complex," D-aspartic acid, the research on these is weak, inconsistent, or shows effects so small they're meaningless. If your testosterone is genuinely low, no over-the-counter booster will fix it. You need bloodwork, lifestyle changes (sleep, body fat reduction, stress management), and possibly actual medical treatment. I wasted hundreds of dollars on these before I understood that.

Fat burners. The main active ingredient in most "fat burners" is caffeine. You can buy caffeine for \$0.03 a pill. The thermogenic effect of these products is clinically insignificant, we're talking maybe 50 to 100 extra calories burned per day, roughly equivalent to a small apple. Some contain synephrine or yohimbine, which can spike your heart rate and blood pressure. Not worth the risk.

Detox teas and cleanses. Your liver and kidneys detox your body 24/7 for free. These products are usually just laxatives and diuretics dressed up in Instagram branding. They make the scale drop temporarily by dehydrating you, then you drink water and it comes right back. Some can genuinely damage your gut lining with long-term use. Skip them entirely.

BCAAs (Branched-Chain Amino Acids). If you eat adequate protein, BCAAs are redundant. The research showing benefits was done on fasted athletes with inadequate protein intake. A scoop of whey protein contains more BCAAs than a scoop of BCAAs, plus the rest of the amino acid profile. This one is pure marketing genius, selling you a fraction of what you'd get from actual protein.

Multivitamins. Controversial take, but the research is clear: for most people eating a reasonably varied diet, standard multivitamins show no measurable benefit in large meta-analyses. They're a safety blanket, not a strategy. Targeted supplementation based on bloodwork is always superior to a scattershot pill.

The pattern here is simple: if a supplement promises dramatic results without effort, it's lying. Save your money for bloodwork and quality food.

GLP-1 Medications: The Tirzepatide Chapter

I'll be straight with you about my experience with tirzepatide (Mounjaro). It changed my life. After years of doing everything "right," tracking calories, exercising consistently, managing stress, and still fighting against a body that seemed determined to hold onto excess fat, tirzepatide was the tool that finally tipped the scales. Literally.

But it's not a magic injection, and the internet doesn't talk enough about the hard parts.

What It Actually Does

GLP-1 receptor agonists (and GIP/GLP-1 dual agonists like tirzepatide) work by mimicking incretin hormones that your gut naturally produces after eating. They slow gastric emptying (food stays in your stomach longer), reduce appetite signals in the brain, and improve insulin sensitivity. Semaglutide (Ozempic/Wegovy) targets GLP-1 only. Tirzepatide targets both GLP-1 and GIP receptors, which may account for its slightly superior results in clinical trials, participants lost an average of 20 to 25% of body weight versus 15 to 17% with semaglutide.

The Side Effects Nobody Warns You About

Nausea. This is the big one. It hit me hard during the first two weeks on each dose increase. It wasn't the kind of nausea where you feel a little off, it was the kind where food becomes genuinely repulsive. What helped: eating smaller, more frequent meals (your stomach empties slower now, so large meals sit there and punish you). Ginger tea, ginger chews, and over-the-counter Pepto-Bismol when it was bad. Injecting at night so the worst of it happened during sleep. Avoiding greasy or heavy foods on injection day. The nausea did fade after 2 to 3 weeks at each new dose for me, but it came back every time I titrated up.

Muscle loss. This is the serious one, and it's the reason I push protein and resistance training so hard throughout this book. GLP-1 medications don't selectively burn fat. When you lose weight rapidly, which these drugs enable, a meaningful portion of that loss is lean mass. Clinical data suggests 25 to 40% of weight lost on GLP-1s can be muscle. That's catastrophic for long-term metabolic health. The fix: eat 0.7 to 1g of protein per pound of bodyweight daily, lift heavy things at least 3 times per week, and lose at a moderate pace. If your doctor starts you on these medications without discussing muscle preservation, find a better doctor.

Constipation or the opposite. Slowed gastric motility affects everyone differently. Some people get constipated (fiber and hydration help enormously). Others get unpredictable GI episodes. Plan accordingly, know where bathrooms are.

Hair thinning. Rapid weight loss of any kind can trigger telogen effluvium, a temporary shedding phase. It happened to me around month three. It recovered. Adequate protein, iron, and biotin help, but mostly it's a patience game.

Sulfur burps. Nobody warned me about these. Rotten egg burps that can clear a room. They're caused by the slowed digestion, food ferments longer in your stomach. Eating smaller meals helps. Some people swear by digestive enzymes. I just suffered through them.

Access and Cost

This is where it gets infuriating. These medications work. They're also expensive (\$800 to \$1,200/month without insurance), inconsistently covered by insurance, and plagued by supply shortages. Compounding pharmacies have filled the gap for some people, but quality varies wildly. Telehealth platforms have made access easier but also created a race to the bottom where

some providers hand out prescriptions with minimal screening.

My advice: work with a doctor who will monitor your bloodwork, adjust your dose based on your response, and actually talk to you about muscle preservation. Don't buy from the cheapest telehealth mill you can find on Instagram.

Your Doctor Relationship Matters More Than Any Supplement

Here's something I wish someone had told me years earlier: stop treating your doctor like a vending machine where you walk in, request a test, and walk out. Find a physician who will partner with you. Someone who looks at your bloodwork trends over time, not just whether you're within "normal" ranges (which are often pathologically broad).

When I started being honest with my doctor about what I was taking, how I was eating, and what I was struggling with, everything got better. She caught a vitamin D deficiency I wouldn't have known about. She flagged my fasting glucose trending upward before it became pre-diabetes. She prescribed tirzepatide with proper monitoring rather than me buying it from some sketchy online pharmacy.

If your doctor dismisses your interest in metabolic health or refuses to run comprehensive bloodwork, find a different doctor. Functional medicine practitioners, sports medicine doctors, and some endocrinologists tend to be more open to proactive optimization rather than just treating disease after it appears. But don't go to the other extreme either, avoid doctors who sell their own supplement lines or push unproven treatments for profit.

The ideal relationship: a doctor who respects your autonomy, runs the tests, interprets the data honestly, and tells you when something is genuinely unnecessary. That person is worth more than any stack of supplements.

Emerging Research Worth Watching

I'm not going to tell you to rush out and try these things. Most are in early research stages. But they're worth knowing about because the science is moving fast.

BPC-157 (Body Protection Compound-157). This is a peptide found naturally in gastric juice. Animal studies show remarkable tissue healing properties, gut lining repair, tendon and ligament healing, neuroprotective effects. The problem: almost all the impressive data is from animal models. Human clinical trials are limited, and the FDA has cracked down on compounding pharmacies selling it. Some people in the fitness and biohacking communities swear by it for injury recovery. I'm watching the research but not personally using it yet. The risk-benefit calculation doesn't pencil out for me until we have better human data.

Semaglutide vs. Tirzepatide. The head-to-head data is becoming clearer. In the SURMOUNT-5 trial, tirzepatide showed roughly 47% greater weight loss than semaglutide over 72 weeks. Tirzepatide's dual mechanism (GLP-1 + GIP) appears to provide better glucose control and weight management for most people. That said, semaglutide has a longer track record, is more widely available, and some people simply tolerate it better. Individual response varies enough that the "best" GLP-1 is the one that works for your body with manageable side effects. Retatrutide, a triple agonist (GLP-1 + GIP + glucagon receptor), is in Phase 3 trials and showing even more impressive results, potentially 24 to 28% body weight loss. This space is evolving fast.

GLP-1s beyond weight loss. Early research suggests these medications may reduce cardiovascular events, improve kidney function, reduce inflammation markers, and potentially protect against neurodegenerative disease. We're in the early innings of understanding what these drugs can do.

The Bottom Line

Supplements fill gaps. They don't replace foundations. Medications can be transformative tools when used under proper medical supervision with clear eyes about side effects. Peptides are an emerging frontier that warrant cautious optimism and rigorous research.

Here's my actual daily stack: creatine (5g), vitamin D3 (4,000 IU with K2), magnesium glycinate (400mg before bed), fish oil (2g EPA+DHA), fiber (10g psyllium husk), and protein powder as needed to hit my target. That's it. No seventeen-pill morning routine. No exotic compounds from the back corners of the internet.

Start with bloodwork. Fix your foundations. Supplement what's actually missing. Find a doctor who gives a damn. And don't let anyone sell you shortcuts, they don't exist, and the people selling them know that better than anyone.

Chapter 8: Know Your Numbers

Everything in this book up to now, nutrition, walking, strength, sleep, even the honest conversation about medications, comes down to one thing. You can't manage what you don't measure. I spent years guessing at my own health based on how my clothes fit and how tired I felt. By the time I actually got bloodwork done, I was already diabetic, with high blood pressure and severe sleep apnea. Those weren't sudden developments. They were years in the making, and I had no numbers to warn me along the way.

Why the Scale Isn't Enough

The scale tells you one thing, and it tells you that one thing poorly. It doesn't distinguish between fat loss and muscle loss. It doesn't tell you whether your blood sugar is trending in the wrong direction, or whether your inflammation markers are creeping up before you feel a single symptom. I know this firsthand. My weight moved plenty over the past year, down to 285, back up to 298, down again to where I am now at 267.8. The number alone never told me the full story of what was actually happening inside my body.

Here's a real example. During the period when I regained weight after Thailand, my scale said I was up 13 pounds. Bad news, right? But when I finally got bloodwork done, my fasting insulin had actually improved because the break from the medication had allowed my body to reset some insulin sensitivity. The weight number was worse. The underlying metabolic marker was better. If I'd been managing by the scale alone, I would have panicked. With the numbers, I could see a more complicated and ultimately more hopeful picture.

Get a Baseline Before You Change Anything

Before you change your diet, start a supplement, or consider a medication, get bloodwork done. Not after you've already started guessing your way through changes, before. Then repeat it every three to six months so you can see what's actually working instead of assuming.

I cannot stress this enough. Your bloodwork is the difference between guessing and knowing. It's the difference between "I think this diet is working" and "My fasting insulin dropped from 18 to 11 and my HbA1c went from 6.2 to 5.4." Those are two very different conversations to have with your doctor.

The Markers Worth Tracking

Here's the list I'd give to anyone starting this process. Not every marker requires the same frequency, but all of them matter at baseline:

Fasting glucose and HbA1c: Your blood sugar control over time. HbA1c reflects your average blood sugar over the past two to three months. Normal is below 5.7 percent. Prediabetic is 5.7 to 6.4. Diabetic is 6.5 and above. Mine was 6.8 when I was diagnosed. Know where you stand.

Full lipid panel: LDL, HDL, and triglycerides. These tell you about your cardiovascular risk. Pay special attention to the triglyceride-to-HDL ratio. A ratio below 2.0 is considered healthy. Above 3.0 is a signal of insulin resistance and elevated cardiovascular risk.

Fasting insulin: Often a better early warning sign than glucose alone, since insulin resistance shows up here before blood sugar itself looks abnormal. This is the marker most doctors don't order unless you ask. Ask. If your fasting insulin is above 10 to 12 uIU/mL, your body is working harder than it should to manage blood sugar, even if your glucose looks normal.

Thyroid panel: TSH and free T3/T4. Thyroid dysfunction is common, often undiagnosed, and directly affects your metabolism, energy, and ability to lose weight.

Vitamin D, B12, and ferritin: Commonly low and easy to miss without testing. Vitamin D deficiency is linked to everything from bone loss to immune dysfunction. B12 deficiency can cause fatigue and cognitive issues, especially in people on metformin, which depletes B12 over time.

hs-CRP: A marker of inflammation. Chronically elevated inflammation is associated with heart disease, diabetes, and cancer. This number gives you a snapshot of what's happening at the cellular level.

Testosterone, or the relevant hormone panel for you: Hormone shifts affect energy, muscle, and fat storage more than most people realize. For men over 40, testosterone levels are worth

monitoring. For women, a broader hormone panel including estrogen and progesterone is important, especially during perimenopause and menopause.

How to Get This Done

You don't necessarily need a full doctor's visit to get a baseline. Plenty of labs and at-home testing services can get you these numbers directly, and then you bring the results to a doctor to interpret and act on.

One service I recommend is SiPhox Health. They offer at-home blood testing kits that cover most of the markers I listed above. You order a kit, do a finger-prick blood sample at home, mail it in, and get your results within days through their online portal. It's affordable, it's private, and it gives you real numbers without needing to schedule a lab visit or fight with insurance for coverage. You can use my affiliate link at siphoxinc.pxf.io/noexcusesjesse if you want to support this work while getting your baseline.

Other options include direct-to-consumer lab services like Ulta Lab Tests, Walk-In Lab, and Quest Diagnostics Direct. All of them let you order bloodwork without a doctor's order in most states. The cost varies, but a comprehensive metabolic panel with lipids and hormones typically runs between 50 and 200 dollars, depending on the service and which markers you include.

Whatever route you take, bring the results to a doctor. The numbers are the starting point. The interpretation and action plan need a medical professional who knows your full history.

Using the Numbers, Not Just Collecting Them

A baseline is only useful if you act on it. If your fasting insulin comes back high, that's a signal to lean harder into the nutrition changes from Chapter 3. If your inflammation markers are elevated, sleep and stress deserve a closer look, not just your diet. If you're considering a GLP-1 medication or a peptide, as I talked about in Chapter 7, your bloodwork is exactly what should be driving that conversation with your doctor, not the other way around.

Here's how I use my own numbers. Every three months, I get a full panel done. I compare it to the previous quarter. I look for trends, not just snapshots. Is my HbA1c moving in the right direction? Is my fasting insulin dropping? Are my inflammation markers stable or improving? If something moves the wrong way, I look at what changed in the previous three months, my nutrition, my

sleep, my stress, my medication, and I adjust.

That's the whole point of this chapter. Nutrition, movement, sleep, and any medication you choose to use, all of it works better when you can actually see what it's doing inside your body, instead of hoping and waiting to see how your pants fit in three months. Know your numbers, and you stop guessing.

The LoadLine Approach

This is what LoadLine Fitness is built around. The LoadLine standard says your weight is one number, but your health is a dashboard. It's not enough to step on a scale. You need to see the full picture: body composition, metabolic markers, cardiovascular indicators, recovery metrics. That's what separates someone who's managing their health from someone who's just watching a number go up and down.

The LoadLine Tracker, the companion tool to this book, was designed to help you track the daily inputs that drive those numbers: your nutrition, your workouts, your sleep, your steps, and your body measurements. It's not a replacement for bloodwork, but it's the daily system that feeds into the quarterly results. You can access it at loadlinefitness.com/tracker.

Chapter 9: Stress, Cortisol, and the Missing Pillar

I want to tell you something that took me years to figure out: you can do everything right with nutrition, movement, strength, and sleep, and still stall out if you're running on stress. I know because I did it. I white-knuckled my way through weeks of clean eating, hit my walking targets, even showed up for strength sessions, and then one bad day at work, one argument, one unexpected bill would send me face-first into a family-size bag of chips or a drive-through order I'd have sworn I'd never touch again.

Here's what nobody told me when I was 317 pounds: stress isn't just an emotional inconvenience. It's a metabolic event. And if you don't manage it, it will manage you.

Cortisol and Your Belly

Let me explain the mechanism without getting too deep into the weeds. When you're stressed, your body releases cortisol. Cortisol isn't evil, it's actually essential. It helps you wake up, respond to threats, and mobilize energy. In short bursts, it's a survival tool. The problem is that modern life doesn't produce short bursts. We produce a slow, endless drip.

Chronic elevated cortisol does several things that work directly against everything in this book. First, it tells your body to store fat, specifically visceral fat, the kind that sits deep in your abdomen around your organs. If you've ever noticed that you tend to gain weight in your midsection first, cortisol is a big part of why. Second, it increases insulin resistance, meaning your cells don't respond as well to insulin, which drives blood sugar up and fat storage higher. Third, it breaks down muscle tissue. So the very muscle you're trying to build with strength training? Cortisol is actively undermining it when you're chronically stressed.

That's not a small problem. That's a wrecking ball aimed at every pillar we've talked about.

The Stress-Sleep-Craving Triangle

Here's where it gets vicious. Stress disrupts sleep. Poor sleep increases cortisol. Elevated cortisol drives cravings, particularly for sugar and high-calorie comfort foods. Those cravings lead to eating that doesn't align with your goals, which creates guilt and more stress. And around you go.

I lived in that triangle for years. I'd lie in bed at night with my mind racing, worries about money, about the business, about whether I was good enough, and I'd end up scrolling my phone until 2 AM. Then I'd sleep poorly, wake up exhausted, drag myself through the day, and by 8 PM my willpower was gone. That's when the real eating started. Not hunger eating. Not even pleasure eating. Stress eating. Filling an emotional hole with food because I hadn't given myself any other way to process what I was feeling.

Breaking the Cycle: My Personal Path

I wish I could tell you I had an epiphany. I didn't. What I had was a slow, uncomfortable realization that I was sabotaging myself. The moment that actually shifted things was almost mundane. I was sitting in my truck after a particularly bad day, fast food bag on the passenger seat, and I caught my reflection in the rearview mirror. I didn't look stressed. I looked defeated. That was the moment I admitted I needed to deal with what was happening in my head, not just what was happening on my plate.

I started small. Impossibly small. Before I ate anything after a stressful moment, I made myself sit in the truck for five minutes. Just five minutes. No phone, no radio, no food. Just breathing. Sometimes I'd be angry. Sometimes I'd be anxious. Sometimes I'd just sit there feeling nothing. But I gave the emotion a moment to exist without shoving food on top of it.

That five minutes turned into a practice. And that practice became one of the most important tools in my toolbox, more important, in some ways, than any supplement or training protocol.

Practical Tools That Work

I'm not going to pretend I'm a therapist or a meditation guru. I'm a 51-year-old guy who used to weigh 317 pounds. But here are the tools I've found that actually move the needle on stress.

Box Breathing. This is the simplest and most effective breathwork technique I've found. Breathe in for four counts, hold for four, breathe out for four, hold for four. Repeat that four times. It takes less than two minutes, and you can do it anywhere, in your truck, at your desk, in the bathroom at a restaurant. What it does physiologically is activate your parasympathetic nervous system, which is the part that tells your body "you're safe, you can stand down." I do this at least three times a day and especially before meals when I notice stress creeping in.

Walking as Stress Relief. We already know walking is one of the four pillars for metabolic health. But I want to highlight its role as a stress tool specifically. There's something about the rhythmic, bilateral movement of walking that processes stress differently than sitting still. I take a twenty-minute walk most evenings, and I don't listen to music or podcasts. I just walk. Some of my best problem-solving happens on those walks, and when I come back, whatever was eating at me feels more manageable.

Journaling. I resisted this one hard. I'm not a "dear diary" kind of guy. But I started keeping a simple log, three things: what stressed me today, how I responded, and what I could do differently tomorrow. Takes five minutes. What it did was help me see patterns I couldn't see when everything was swirling around in my head. I realized that most of my stress eating happened on days when I felt out of control, and that I had specific triggers I could prepare for.

Setting Boundaries. This was the hardest one. I had to learn to say no. To people, to commitments, to the voice in my head that said I had to do everything for everyone. Setting boundaries isn't selfish, it's survival. If your stress load is too high, something has to give. Better it be an optional commitment than your health.

When to Get Professional Help

Here's where I need to be straight with you. The tools I just described are good, and they helped me a lot. But they have limits. If you're dealing with chronic anxiety, depression, trauma, or

emotional eating that feels compulsive and uncontrollable, please talk to someone. A therapist, a counselor, a psychologist, there's no version of this where powering through alone makes you tougher.

I waited too long to consider it. I told myself I was fine, that I just needed more discipline. What I actually needed was a professional who could help me understand why I ate the way I ate when I was hurting. When I finally did that work, and I did it with a licensed counselor, it didn't weaken my other pillars. It made all of them stronger.

If cost is a barrier, look into community mental health resources, sliding-scale providers, or telehealth options. Affordable counseling exists. Don't let money be the excuse you use not to get help.

Why This Is the Invisible Fifth Pillar

I call stress management the invisible fifth pillar because you can't see it. You can see someone's nutrition. You can watch them walk. You can track their sleep. But stress lives underneath all of that, quietly pulling strings. A person can follow every recommendation in this book and still fail if their stress load is crushing them.

I also call it invisible because we don't talk about it. In fitness culture, we talk about willpower, discipline, and pushing through. We don't talk about the fact that a person working two jobs, raising kids, and carrying debt is operating in a fundamentally different metabolic state than someone with flexibility and support. Stress isn't a character flaw. It's a biological reality, and it deserves to be treated as seriously as your protein intake or your step count.

So here's my ask: take your stress management as seriously as you take the other pillars. Not more seriously, this isn't about replacing action with introspection. It's about recognizing that the external work and the internal work are the same work. You cannot build a strong body on a foundation of chronic stress any more than you can build a house on sand.

Take a breath. Go for a walk. Write down what's eating you. And if you need help, go get it. There's no shame in that. There never was.

Chapter 10: The Female Factor

I need to draw a line before I start this chapter: I'm a man. I've never experienced perimenopause, menopause, or the hormonal shifts that women navigate in their forties and fifties. Everything I'm about to write comes from what I've learned coaching women, listening to female clients and colleagues, and doing my best to understand a process I will never personally live through. So if some of this feels secondhand, that's because it is. I have enormous respect for the women who've shared their experiences with me and helped me understand what they're dealing with.

That said, I wrote this chapter because too many books and programs either ignore women's hormonal health entirely or treat it as a footnote. It's not a footnote. For millions of women over forty, hormonal change is the central challenge of their metabolic health journey, and it deserves a real conversation.

What Happens During Perimenopause and Menopause

Perimenopause, the years leading up to menopause, typically starts in a woman's forties but can begin earlier. During this time, estrogen and progesterone levels fluctuate erratically and eventually decline. Menopause itself is reached when menstruation has stopped for twelve consecutive months.

The metabolic consequences of this shift are significant and often underestimated. Estrogen plays a role in how the body distributes fat, when levels drop, fat tends to shift from the hips and thighs toward the abdomen. Sound familiar? It's the same visceral fat pattern we talked about in the cortisol chapter. Except now it's being driven by a hormonal change that no amount of willpower can override.

Sleep disruption is another major consequence. Hot flashes, night sweats, and changes in sleep

architecture mean that many women in perimenopause and menopause sleep poorly, not because of bad habits, but because their bodies are fighting them. And as we've discussed throughout this book, poor sleep drives everything else: worse insulin sensitivity, stronger cravings, reduced recovery.

Cravings also shift. Some of this is biological, fluctuating hormones can drive sugar cravings directly. Some of it is compensatory, women who aren't sleeping well crave quick energy. And some of it is emotional, because the perimenopause transition often coincides with major life stressors: kids leaving home, aging parents, career pressures, relationship changes. It's a lot.

Why Protein Needs Increase

One of the most important adjustments for women over forty is increasing protein intake. There are several reasons for this. First, muscle protein synthesis becomes less efficient with age, you need more protein to trigger the same muscle-building response you got easily at twenty-five. Second, declining estrogen accelerates muscle loss, which accelerates metabolic decline, which accelerates fat storage. It's a domino chain.

I coach women toward 0.7 to 1 gram of protein per pound of body weight, which is the same range I use for men. Some women push back on this because they've been taught to eat less, not more. But protein is the macronutrient that supports muscle, regulates appetite, and helps manage blood sugar. For a woman navigating perimenopause, eating enough protein isn't optional, it's foundational. If you're going to change one thing about your diet, make it this.

Strength Training for More Than Muscle

We've talked about strength training as a pillar throughout this book. For women over forty, it's even more critical because of bone density. Estrogen is protective of bone health. When estrogen declines, bone loss accelerates, increasing the risk of osteoporosis. The single best defense against this? Resistance training.

Strength training stimulates bone formation through mechanical loading. It also builds the muscle

that supports joints, improves balance, and reduces fall risk. I've had female clients tell me they're afraid of getting "bulky" from lifting weights. I'll be honest: as a 51-year-old man who lifts regularly, building visible muscle mass is hard enough. It's not something that happens accidentally. What does happen, reliably, is stronger bones, better posture, improved insulin sensitivity, and a body that feels more capable. Every woman I've trained who committed to strength work has told me the same thing: "I wish I'd started this ten years ago."

Adjusting the Four Pillars Framework

The four pillars, nutrition, movement, strength, and sleep, apply to everyone. But for women navigating hormonal change, the emphasis shifts:

Nutrition: Prioritize protein above all else. Consider working with a registered dietitian who specializes in midlife women's health. Track how different foods affect your energy and mood, because hormonal sensitivity can change tolerances.

Movement: Walking is still king. But add more intentional movement throughout the day. Sedentary behavior hits harder when your baseline metabolic rate is declining.

Strength: Two to three sessions per week. Focus on compound movements, squats, deadlifts, rows, presses. Progressive overload matters more than fancy programming. Lift heavy enough that it's challenging. Your bones are listening.

Sleep: This is where I expect the most resistance, because perimenopausal sleep disruption isn't always fixable with good habits alone. Blackout curtains, a cool room (65 degrees F), consistent bedtimes, these help. But if sleep is consistently poor, that's a conversation worth having with a doctor, potentially about hormone replacement therapy or other medical interventions. I don't prescribe anything, but I do advocate for exploring every option with qualified professionals.

What I've Learned Coaching Women

If I'm being honest, the biggest thing I've learned is humility. The strategies that worked for me, a man losing weight in his late forties with tirzepatide, don't transfer one-to-one. Women's bodies

are playing a different game with different rules. Hormonal fluctuations create bad days through no fault of their own, and I've learned not to treat those days as compliance failures.

I've also learned to shut up and listen. When a woman tells me her body feels different, it does. When she says she can't sleep despite doing everything right, there's probably a hormonal reason. When she says the cravings feel like they're coming from somewhere deeper than hunger, they probably are. My job isn't to explain away her experience. It's to work with it.

One more thing: I've watched women do this work, navigating hormonal challenges while building stronger bodies and better metabolic health, and their resilience puts mine to shame. The perimenopause transition is no joke. It affects sleep, mood, body composition, energy, and identity all at once. Doing the work of strength training and nutrition management on top of that? That takes a kind of grit that the fitness industry doesn't celebrate enough.

Resources and Encouragement

If you're a woman reading this book and recognizing your story in these words, here's what I want you to know: you are not broken. Your body is going through a transition, not a decline. The tools in this book, protein, strength training, walking, sleep hygiene, are not just helpful, they're essential. They are your best defense against the metabolic slowdown that comes with hormonal change.

Seek out practitioners who understand midlife women's health. Not all doctors are well-versed in perimenopause management, and you may need to advocate for yourself. Organizations like The Menopause Society and women's health-focused dietitians and trainers are valuable resources.

And please don't let anyone tell you it's "just age." It's not just age. It's biology, and biology responds to intervention. You have more control than you've been led to believe. Not perfect control, none of us have that, but enough to get stronger, healthier, and more resilient at fifty than you were at forty.

I believe that. And if you do the work, you'll prove it to yourself.

Chapter 11: Putting It All Together

By now you've got the pillars laid out separately: nutrition, movement, sleep, knowing your numbers, stress management, and for many of you, the hormonal considerations that change how all of this applies to your body. The question I get asked most often, and the one I asked myself for a long time, is simple: where do I actually start, and how does this fit into a normal week?

You Don't Start All at Once

I want to say this clearly, because it's the mistake I made more than once. Trying to overhaul your eating, start walking daily, build a strength routine, fix your sleep, manage your stress, and get bloodwork done all in the same week is how good intentions collapse by day four. Pick one pillar, build a small version of it for two to three weeks, then layer the next one on top. This is slower than it feels like it should be. It's also the version that actually survives past the first month.

Here's the sequence I recommend, based on my own experience and what I've seen work for the people I've coached:

Phase 1 (Weeks 1-2): Nutrition basics and walking

Phase 2 (Weeks 3-4): Add the band circuit

Phase 3 (Weeks 5-6): Lock in sleep routine

Phase 4 (Weeks 7-8): Add stress management practices

Phase 5 (Week 9+): Get bloodwork and refine based on data

Each phase adds one new pillar on top of the ones already in motion. By week nine, you're doing all five, but you never had to build more than one at a time.

A Simple Weekly Rhythm

Once you've got a pillar or two in motion, here's a rhythm that holds them together without requiring you to think hard about it every day:

Daily:

- Protein at every meal (25 to 40 grams per meal)
- A post-meal walk after your largest meal
- A consistent wake time
- Water throughout the day (half your bodyweight in ounces)
- Five to ten minutes of stress reduction (breathwork, quiet time, or a short walk)

Three Times a Week:

- The stretch and band circuit from Chapter 5 (30 to 45 minutes)

Ongoing:

- Building toward your daily step target from Chapter 4
- A wind-down routine before bed (no screens, dim light, 30 to 60 minutes)

Every Three to Six Months:

- Bloodwork, so you're adjusting based on real numbers instead of guessing

That's the whole structure. It's not complicated, and it's not supposed to be. Complexity is usually what kills a plan before month two.

A 30-Day Starting Point

If you want a concrete place to begin, here's how I'd sequence the first month:

Week 1: Protein at every meal, and a short walk after dinner. Nothing else changes yet. This is your foundation. If you do nothing else this week, get 25 to 40 grams of protein at each meal and

take a 10-minute walk after your biggest meal.

Week 2: Add the post-meal walk after your other two meals as well, and cut liquid sugar, soda, juice, sweetened coffee drinks. You should be hitting 6,000 to 7,000 steps by now. Keep the protein going.

Week 3: Add the stretch and band circuit, three times this week. Start with the bodyweight and light band versions from Chapter 5. Your body needs time to adapt to the new movement patterns.

Week 4: Lock in your wind-down routine and a consistent wake time, and schedule your baseline bloodwork if you haven't already. Set a specific bedtime and wake time. Hold them within a 30-minute window every day.

By the end of thirty days, you're not doing everything perfectly. You're doing four real things consistently, and that's a completely different outcome than trying everything at once and burning out by week two.

The LoadLine Standard in Practice

This is where the LoadLine framework comes together. The LoadLine standard says:

TRAIN: Three resistance sessions per week, using the band circuit or a gym program.

MOVE: 7,000 to 10,000 steps daily, with post-meal walks.

RECOVER: Seven to nine hours of sleep, a wind-down routine, and stress management.

TRACK: Daily logging of nutrition, workouts, steps, and body measurements. Quarterly bloodwork.

REPEAT: Consistency over intensity. Every day. Every week. Every month.

You don't need to be perfect at any one of these. You need to be consistent at all of them. A B-minus effort across all four pillars beats an A-plus effort on one pillar and neglect of the other three. That's the whole philosophy.

Using the LoadLine Tracker

The LoadLine Tracker, available at loadlinefitness.com/tracker, was built to support exactly this system. It tracks your daily nutrition (the 14-day meal plan from Chapter 3), your workouts (the band circuit with progression tracking), your supplements, your steps, and your body measurements. It calculates your compliance score across all domains and shows you trends over time.

You don't need the tracker to make this work. A notebook and a pen will do the same job. But if you want a tool that was built around this specific system, it's there.

Expect Setbacks, Plan for Them

You will miss a walk. You will have a week where sleep falls apart because life happens. You'll eat something that doesn't fit the plan. That's not a reason to start over from zero, and it's not a sign the system failed. It's a normal week.

Here's my rule: a missed day doesn't become a missed week. If you miss Monday, you show up Tuesday. You don't wait until next Monday to restart. You don't punish yourself with a stricter plan. You just pick back up at whichever pillar slipped, and keep going.

The version of this that works long term isn't the one where you never miss a day. It's the one where a missed day doesn't turn into a missed month. That's the difference between someone who maintains their results and someone who ends up back at the starting line.

The Compounding Effect

I want you to understand something that won't be obvious in the first few weeks. These pillars compound. Nutrition improves your energy, which makes walking easier. Walking improves your blood sugar, which makes cravings more manageable. Better sleep improves your recovery, which makes the band circuit feel easier. Lower stress improves your sleep, which gives you more willpower for nutrition.

Each pillar doesn't just add to the others. It multiplies them. That's why a mediocre effort across all four beats a heroic effort on one. The system works as a system, not as a collection of separate habits.

I noticed this myself around month three. I wasn't doing anything dramatically different from

month two. But the combination of consistent protein, consistent walking, consistent sleep, and consistent strength work had started to produce results that were clearly larger than the sum of the parts. My energy was different. My cravings were different. My body was different. Not because I was working harder, but because the system had been running long enough to compound.

Give it time. The results come.

Chapter 12: Where to Go From Here

If you've read this far, you already have everything you need to start. Nutrition, movement, sleep, stress management, knowing your numbers, laid out the honest way, without the diet culture noise and without pretending a medication or a peptide replaces the work underneath it. That was always the point of this book, to give you the real version of what it takes, the version I wish someone had handed me back in October 2025 instead of making me learn it the hard way.

What a Book Can and Can't Do

A book can give you the map. It can't walk the road with you, adjust when your week gets thrown off, or look at your actual bloodwork and tell you what it means for you specifically. That's not a limitation of this book, it's just the honest difference between information and coaching. Plenty of people will read this and build exactly what they need on their own. Others will want someone in their corner while they do it, the way I eventually built that for myself through trial, error, and a lot of notes I kept along the way.

There's also an accountability gap that information alone can't close. Knowing what to do and actually doing it consistently for months are two very different things. The research on behavior change consistently shows that accountability, someone checking in, someone who expects you to show up, is one of the strongest predictors of long-term success. You can get that from a friend, a spouse, an online community, or a coach. The source matters less than the consistency.

Where LoadLine Fitness Comes In

Everything in this book grew out of what became LoadLine Fitness, and the LoadLine standard, built from actually living this process rather than designing it from a whiteboard. If you want to go further than this book alone, that's what it's there for, real coaching, a structure built around the same pillars, and someone checking in on the numbers with you instead of you tracking them alone in a notebook.

The LoadLine community is growing. We have people at every stage of this process, from someone just starting at 300+ pounds to people who've been at their target for months and are focused on performance and optimization. What we share is the same framework: the LoadLine standard, the same pillars, and the same commitment to managing health with data instead of guesswork.

The No Excuses Ninety

The No Excuses Ninety program takes everything in this book and turns it into a guided ninety-day path. It's built around the same four pillars, but with structure, accountability, and real-time adjustments that a book can't provide.

Here's what it includes:

Week 1-2: Assessment and Foundation. You get a full baseline bloodwork panel, body measurements, and a detailed intake that looks at your current nutrition, movement, sleep, and stress. We build your personalized starting plan based on where you actually are, not where a generic program assumes you should be.

Week 3-12: The Four Pillars in Motion. Each two-week phase introduces one new pillar, exactly like the sequencing in Chapter 11, but with weekly check-ins, plan adjustments based on your progress, and direct access to coaching when you hit a wall.

Week 13: Post-Program Bloodwork and Review. You get your bloodwork done again at the end of ninety days so you can see exactly what changed inside your body, not just what changed on the scale.

Throughout the program:

- Weekly accountability check-ins
- A customized nutrition framework based on your preferences and bloodwork
- A progressive movement plan that scales with your capacity
- Sleep and stress management guidance tailored to your situation
- Access to the LoadLine community

The program isn't cheap, and it isn't easy. But it's built on the same system that took me from 317 pounds to where I am now, and it's the version with the guardrails that keep you from falling into the traps I fell into on my own.

If you're interested, visit loadlinefitness.com/365 or reach out through the No Excuses Jesse social channels. We'll have a conversation about whether it's the right fit for you.

The Community You Didn't Know You Needed

One thing I didn't talk about enough in this book is the role of community. When I started No Excuses Jesse, I did it to hold myself accountable. What I found was that the community that grew around it, the people who followed along, shared their own stories, and pushed me to keep going when I wanted to coast, became one of the most important pillars of my own progress.

You don't need a large community. You need one or two people who know what you're working toward and who will tell you the truth when you start making excuses. That's what No Excuses Jesse is. That's what LoadLine Fitness is. We're not a cheerleading squad. We're a group of people doing the work and holding each other to it.

Find your version of that. Whether it's us, or a local group, or a friend who's willing to walk with you three mornings a week. The research on social support and health outcomes is overwhelming: people with accountability partners are significantly more likely to maintain long-term behavior change than people going it alone.

The Last Thing I Want You to Take From This

Go back to the line that closed Chapter 1. Fix the habits, or the weight always finds its way back. I've lived both sides of that sentence, the version where a medication did the work alone and it came right back, and the version where I finally built something underneath it. The second version is slower. It's also the only one that lasts.

You don't need to be perfect starting today. You need to start. One pillar. One habit. One walk. One meal with real protein. One night of actual sleep. Stack those, week after week, month after month, and the compound effect does what no diet, no medication, and no amount of willpower can do on its own.

This is the honest version. The real version. The version that actually works.

No excuses.

Appendix A: Sources and Further Reading

Chapter References

Chapter 2: Why Willpower Isn't the Problem

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Chapter 3: Nutrition, Without the Diet Culture

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- USDA Dietary Guidelines for Americans, 2020-2025. (General nutrition framework)

Chapter 4: Just Get Up and Walk

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Chapter 5: Strength Without the Gym

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Chapter 6: Sleep Is the Multiplier

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- Spiegel, K., et al. (2004). "Brief communication: Sleep curtailment in healthy young men is associated with decreased leptin levels, elevated ghrelin levels, and increased hunger and appetite." *Annals of Internal Medicine*, 141(11), 846-850. (Sleep and hunger hormones)
- Walker, M.P. (2017). *Why We Sleep: Unlocking the Power of Sleep and Dreams*. New York: Scribner. (Comprehensive sleep science)

Chapter 7: Supplements, Medications, and Peptides

- Kreider, R.B., et al. (2017). "International Society of Sports Nutrition position stand: Safety and efficacy of creatine supplementation in exercise, sport, and medicine." *Journal of the International Society of Sports Nutrition*, 14, 18. (Creatine safety and efficacy)
- Wilding, J.P.H., et al. (2021). "Once-weekly semaglutide in adults with overweight or obesity." *New England Journal of Medicine*, 384(11), 989-1002. (STEP 1 trial, GLP-1 efficacy)
- Jastreboff, A.M., et al. (2022). "Tirzepatide once weekly for the treatment of obesity." *New England Journal of Medicine*, 387(3), 205-216. (SURMOUNT-1 trial, tirzepatide efficacy)

Chapter 8: Know Your Numbers

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- American Diabetes Association. (2024). "Standards of Medical Care in Diabetes." *Diabetes Care*, 47(Supplement 1). (HbA1c diagnostic criteria)

Chapter 9: Stress, Cortisol, and the Missing Pillar

- Epel, E., et al. (2000). "Stress and body shape: Stress-induced cortisol secretion is consistently greater among women with central fat." *Psychosomatic Medicine*, 62(5), 623-632. (Cortisol and abdominal fat)
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Chapter 10: The Female Factor

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- Maltais, M.L., et al. (2009). "Changes in muscle mass and strength after menopause." *Journal of Musculoskeletal and Neuronal Interactions*, 9(4), 186-197. (Muscle loss in menopause)
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Additional Reading

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- Lustig, R. (2012). *Fat Chance: Beating the Odds Against Sugar, Processed Food, Obesity, and Disease*. New York: Hudson Street Press. (Sugar and metabolic health)
- Matthew Walker. (2017). *Why We Sleep*. New York: Scribner. (Sleep science)
- Peter Attia, MD. (2023). *Outlive: The Science and Art of Longevity*. New York: Harmony Books. (Longevity and metabolic health)

Appendix B: Quick Reference Guide

Daily Checklist:

- [] Protein at every meal (25-40g per meal, 0.7-1g per lb of target bodyweight daily)
- [] Post-meal walks (10-15 min after each meal)
- [] Hydration (half your bodyweight in ounces)

- ☐ Consistent wake time (within 30-min window)
- ☐ Wind-down routine (30-60 min before bed, no screens)
- ☐ Log your numbers (weight, waist, meals, workouts, steps)

Weekly Checklist:

- ☐ 3x resistance training (band circuit or gym)
- ☐ 7,000-10,000 steps most days
- ☐ Review your week (what worked, what didn't)
- ☐ Prep protein sources for the week ahead

Monthly Checklist:

- ☐ Body measurements (weight, waist, photos if desired)
- ☐ Review trends in your tracker data
- ☐ Adjust plan based on progress
- ☐ Order supplements if needed

Quarterly Checklist:

- ☐ Full bloodwork panel (see Chapter 8 for markers)
- ☐ Compare to previous quarter
- ☐ Review and adjust nutrition, training, and supplementation with your doctor
- ☐ Set goals for the next quarter

Protein Quick Reference:

- 1 chicken breast (6 oz cooked): ~42g protein
- 1 can of tuna: ~25g protein
- 6 oz salmon: ~34g protein

- 1 cup Greek yogurt: ~20g protein
- 1 cup cottage cheese: ~25g protein
- 3 whole eggs: ~18g protein
- 1 scoop protein powder: ~25g protein
- 1 cup black beans: ~15g protein
- 4 oz lean ground turkey: ~28g protein

Step Count Targets by Starting Level:

- Currently under 3,000/day: Target 4,000 by end of month 1
- Currently 3,000-5,000/day: Target 7,000 by end of month 1
- Currently 5,000-7,000/day: Target 9,000 by end of month 1
- Currently 7,000+: Target 10,000+ and maintain

Band Circuit Quick Reference (from Chapter 5):

Exercise | Sets | Reps | Notes

Band pull-aparts | 3 | 15 | Posture, upper back

Banded squats | 3 | 12-15 | Band above knees

Banded rows | 3 | 12/side | Anchor and pull

Banded glute bridges | 3 | 15 | Band above knees

Standing chest press | 3 | 12 | Anchor behind

Banded lateral walks | 2 | 10/direction | Hip stability

Band bicep curls | 3 | 12-15 | Elbows at sides

Banded overhead press | 3 | 10-12 | Full lockout

Sleep Hygiene Checklist:

- ☐ Same wake time daily (within 30 min)
- ☐ Morning sunlight within 1 hour of waking (5-10 min)
- ☐ No caffeine after 1-2 PM
- ☐ Dim lights 60 min before bed
- ☐ No screens 30-60 min before bed
- ☐ Room temperature 65-68F
- ☐ Dark room (blackout curtains or sleep mask)
- ☐ Quiet environment (earplugs if needed)

Box Breathing (for stress management):

1. Inhale through nose for 4 seconds
2. Hold for 4 seconds
3. Exhale through mouth for 4 seconds
4. Hold for 4 seconds
5. Repeat 4-8 cycles
6. Practice 2-3 times daily or during stressful moments

Resources:

- LoadLine Fitness: loadlinefitness.com
- LoadLine Tracker: loadlinefitness.com/tracker
- LoadLine Solo (Self-Paced Program): loadlinefitness.com/solo
- LoadLine 365 (Coaching Program): loadlinefitness.com/365
- SiPhox Health (At-Home Blood Testing): siphoxinc.pxf.io/noexcusesjesse
- No Excuses Jesse on Instagram: [@noexcusesjesse](https://www.instagram.com/noexcusesjesse)

- No Excuses Jesse on TikTok: @noexcusesjesse

About the Author

No Excuses Jesse is the public identity of Jesse Collins, founder and CTO of LoadLine Fitness & Wellness, based in Queen Creek, Arizona. At 51 years old, Jesse went from 317 pounds, diabetic, hypertensive, and unable to walk to his mailbox without resting, to building a comprehensive health system based on the same principles in this book.

LoadLine Fitness was born from Jesse's own journey and his frustration with an industry that sells quick fixes instead of sustainable systems. The LoadLine standard, TRAIN, MOVE, RECOVER, TRACK, REPEAT, is the framework that emerged from months of personal experimentation, failure, and refinement.

Jesse holds no formal credentials in nutrition, exercise science, or medicine. What he has is lived experience, the data from his own bloodwork and body composition, and a coaching framework built from the ground up by someone who actually lived the problem. He is not a replacement for your doctor. He is the guy who kept notes when nobody else was writing them down honestly.

For more information about LoadLine Fitness, coaching programs, and the LoadLine community, visit loadlinefitness.com or find No Excuses Jesse on social media.